Encompass Health (NYSE: EHC)

Price Target: $90
19% 2-Year IRR

Healthcare (HC) Group

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October 18th, 2023

Western Investment Club

Opportunity Overview

Encompass Health

Buy at $63 – Strong operational history with utilization tailwind and margin expansion story

Opportunity Overview

Highlights

- EHC owns and operates 158 inpatient rehabilitation facilities ("IRFs") across 37 states and Puerto Rico. IRFs are post-acute sites of care with acute care hospitals being the source for 90%+ of admissions
- The market has fallen behind on EHC's secular growth story, given strong performance in healthcare services, investors have focused on hospitals and MCOs
- EHC benefits from strong discharge momentum, 110 planned bed additions for H2 '23, bed level IRRs are very attractive
- Guidance appears to be conservative, Q2 '23 EBITDA beat by +15% while guidance raised by +4% at mid-point. Attractive setup for FY23 beat given combination of utilization tailwinds and conservative guide (1Q '23 beat by +10% and narrowed guide by +1% at mid-point), mindful of tougher comps in H2 '23

Catalysts:

- **Earnings Calls:** EHC earnings calls provide significant insight, significant read through from MCO commentary on MLRs and Medicare utilization, directional outlook on FY24 discharge growth
- **Market Share Capture:** Secular shift in site of care and value proposition should get investors excited about the stock. Medicare Advantage payors have indicated that IRFs are increasingly attractive vs SNFs and LTACHs

Thesis Overview

Thesis:

- **Attractive supply-demand dynamic:** EHC is the best way to play strong growth in the Medicare population (core cohort growing +5% per year for N5Y). IRF supply has stayed flat over the past 20 years, wave of Medicare beneficiaries in need of post-acute care will visit EHC facilities
- **EHC is the highest quality platform:** IRFs benefit from scale, margins are attractive when facility size is above 40 beds (EHC average facility has ~67 beds), EHC is a specialized operator of IRFs, receives minimal outlier payments while providing the best patient care
- **Strong utilization trend and long-term share capture:** EHC has delivered four consecutive quarters of accelerating SSS discharge growth, MCO commentary read through suggests continued momentum through FY24. EHC has captured share from competing post-acute sites of care, IRF value proposition resonates with payors
- **Attractive margin expansion story as contract labour utilization eases:** EHC hired contract labour to meet demand during the pandemic, saw margin compression like many other providers, as contract labour inflects downwards, expect margin uplift of ~75 bps plus operating leverage benefits (glide path to low-20s EBITDA – NCI margin)

Investment Summary

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Encompass Health is a compelling healthcare services long idea

Clear Positives

- Specialized portfolio of healthcare assets led by management with serious operational chops, captured share over time from competing IRFs
- Long-term tailwind of positive demographic growth should allow for strong discharge performance
- EHC has turned the corner on numerous macro issues facing providers including labour woes, sequestration, home health transfer exemption, the company has a straightforward operational path ahead
- Staffing disruptions have turned the corner save for another rampant COVID surge that could disrupt labour or bed capacity
- Facility level IRRs are attractive, the company generates 30%+ annual returns on bed additions and 12-13% on new facilities
- Medium term utilization tailwind evidenced by MCO commentary, strong discharge growth through FY24
- Modest leverage profile of 3.0x debt to EBITDA with capacity for additional M&A
- Recession resilient discharge growth, grew SSS organically through the GFC, limited downside in recession case

Clear Negatives

- Sequestration resumption applies pressure on revenue/discharge growth
- Margin expansion post contract labour reversal solely driven by volumes
- Multiple expanded over the last year, however, not completely full given simplified operational ahead
- Tougher comps in H2 '23, +16.5% EBITDA growth in Q4 '22
- Facility IRRs are in 12-13% range, higher construction costs effect on long-term IRRs

Investment Risks

- Home health transfer exemption risk – price cuts for IRFs could hurt EHC
- Stroke of pen risk – reimbursement completely in the hands of the CMS
- Labour woes continue to compress margins, COVID surge hurts bed utilization
- Costs per discharge grows more than revenue per discharge, margin growth driven strictly by volume

Key Alignment Criteria

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Outlook on key drivers that investors care about

Criteria	Outlook	Diligence
Utilization Trend	• Strong utilization trends in the near-term evidenced by MCO commentary • 4 quarters of accelerating SSS growth, exit rate of ~10% for Q2 '23 • Market volume growth of 4-5% driven by Medicare beneficiary age cohort, above market growth for EHC due to market share gains	
Labour Outlook	• Alt data (although sell-side furnished) indicates that EHC has turned the corner on its labour woes • Management commentary on hiring and retention strategy makes helps alleviate margin pressures • Sell-side models are overly conservative on margin improvement	
Reimbursement Risk	• CMS and MCO bids determine reimbursement for a provider in any given year, investors must be comfortable with stroke of pen risk • ~22% of the industry is upside down on margins, decreasing payment for IRFs (below inflation market basket updates or home health transfer exemption) puts 1/4th of the industry by volume under more pressure	
Valuation	• Trades at 10.2x FY23E EBITDA – NCI, given the setup with conservative guidance and utilization trends, investors aren't paying a premium multiple • HC see's the multiple as being nearly full, likely some upside given EHC trades at 60th percentile of its multiple range with a clear operational path forward and increasing value of market share gain	

Facility Overview

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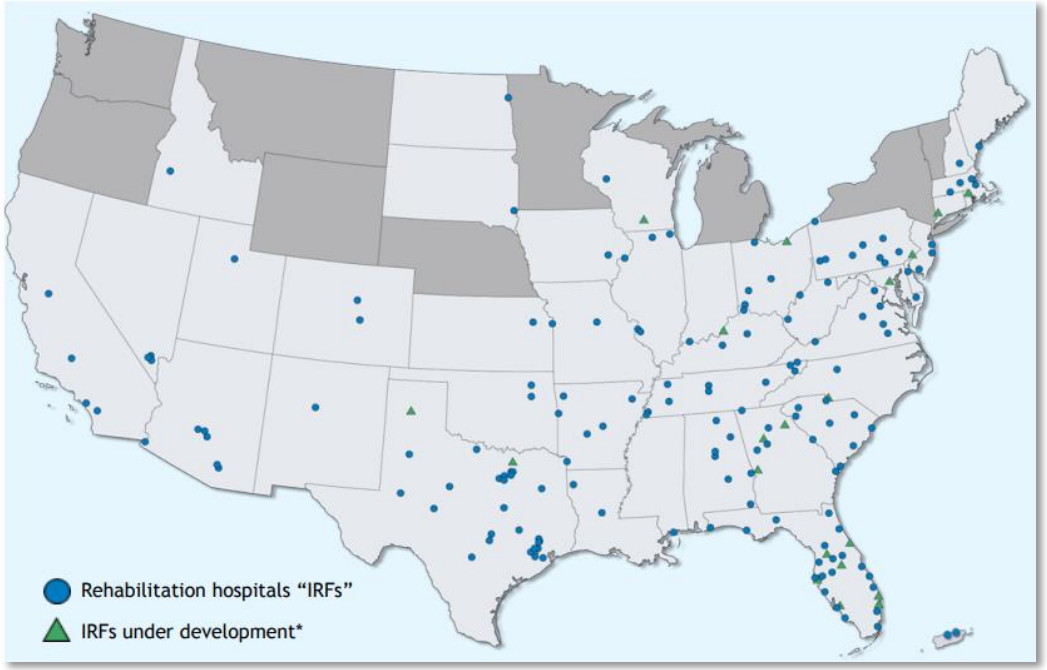
EHC's Inpatient Rehabilitation Facility footprint



IRFs are post-acute sites of care, avg. patient age is 71. Patients deemed unfit to go home after a discharge from a hospital can visit an IRF

158 Inpatient Rehabilitation Facilities (21 Total in Pipeline)

61 IRFs through JVs (5 JVs in Pipeline)

10,611 Total Licensed Beds in 2Q23 (72.4% Occupied)

~12.3 Days Average Length of Stay in 2Q23

EHC owns and operates 158 IRFs across 37 states and Puerto Rico

Business Primer

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Encompass Health is an attractive facilities investment opportunity

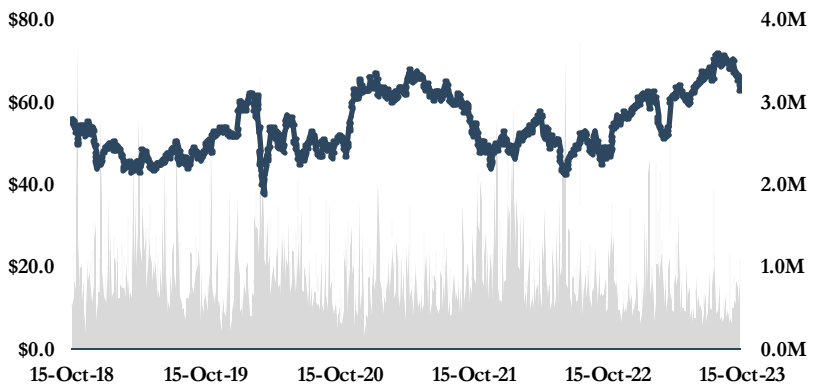
Business Primer

- EHC is the largest owner and operator of IRFs (inpatient rehabilitation facilities) in North America; 25% of licensed beds are inside EHC facilities, certain states have CON requirements for facility builds
- IRFs are post-acute facilities for treatment of patients discharged from acute care hospitals who aren't ready to be discharged to the community, EHC competes with freestanding IRFs, IRFs owned by acute care hospitals, SNFs, LTACHs and home-health agencies
- CMS requires that 60% of an IRF's admissions must have at least one medical diagnosis or functional impairment from a list of 13 compliant conditions ("CMS-13")
- Post-acute is an attractive industry to invest in, demographic trends are a tailwind for discharge growth (5% 4-year population CAGR for patients in EHC's average patient age cohort); number of IRF facilities has stayed flat over L10Y making for an attractive supply-demand dynamic
- EHC is the highest quality platform in the industry, IRF margins benefit from scale, four wall EBITDA margins are 20%+ for hospitals with 50+ beds; margins are much lower for smaller players
- EHC grew discharge volumes organically through the GFC, input SW&B are ~55% of revenue, labour costs inflated through the pandemic (driven by contract labour hiring and wage inflation), recession case eases labour woes without hurting volumes
- Strong near-term outlook on discharge growth driven by normalization of utilization (evidenced by MCO commentary), long-term outlook driven by share capture within post-acute and margin upside

Key Financial Data

Share Price	$63	Revenue	$	Multiple
DSO	100	FY23E - RM	$4,749	1.9x
Market Capitalization	**$6,355**	FY23E - Consensus	$4,781	1.9x
(+) Long Term Debt	2,939			
(-) Cash	118	EBITDA		
Total Enterprise Value	**$9,176**	FY23E - RM	$948	9.7x
		FY23E - Consensus	$937	9.8x
Dividend Payout	0.9%	FY24E - RM	$994	9.2x
Equity Beta	0.9	FY24E - Consensus	$1,012	9.1x
ADV	0.6			

Stock Price Action



IRF Payment Value Chain

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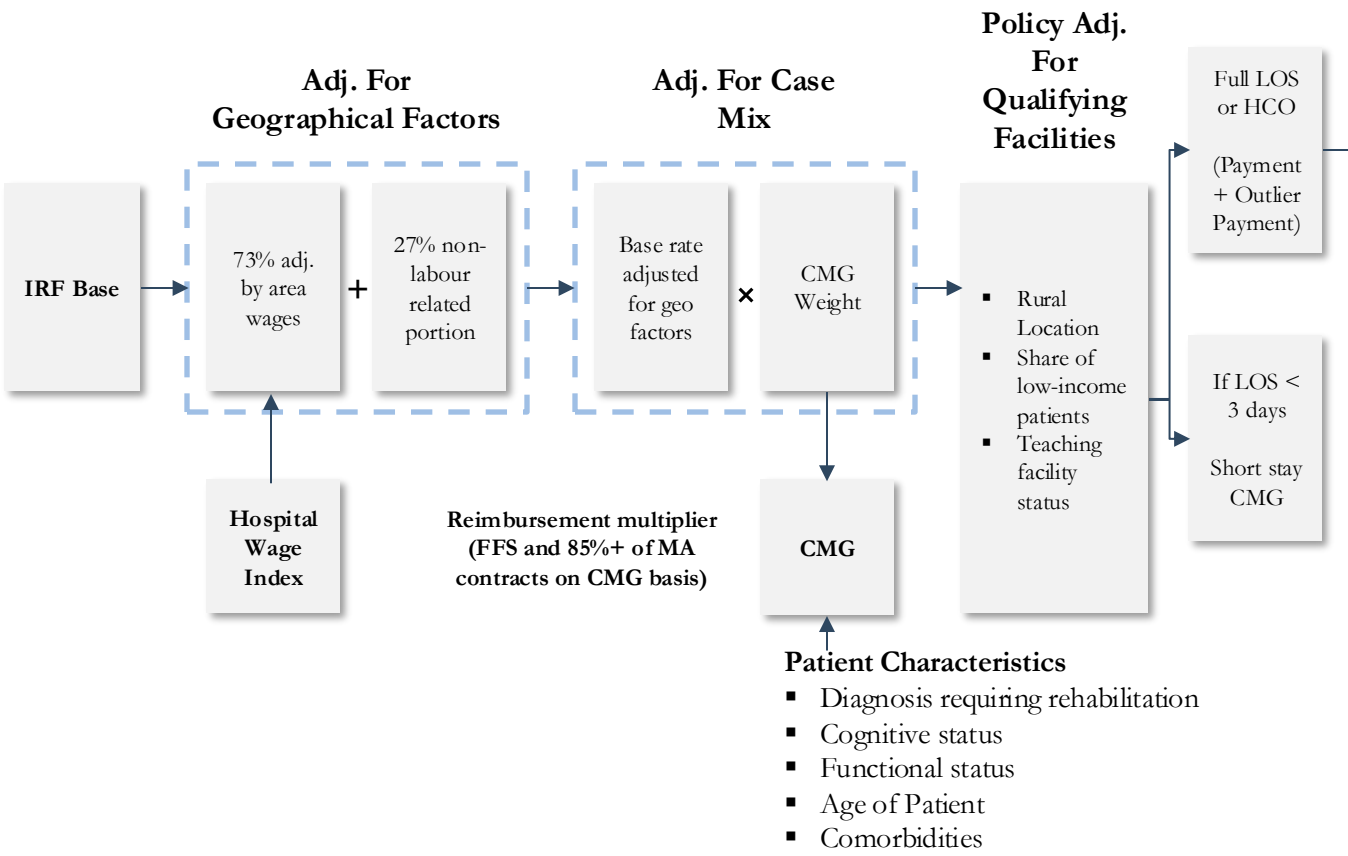
Breaking down reimbursement for Inpatient Rehabilitative Facilities

IRF Payment Chain



Outlier Payments

EHC specializes at managing IRFs and CMS-13 patients, as a result, EHC treats 30% of all Medicare discharges but only makes up 4% of the outlier payments. EHC's value proposition resonates with payors and leads to share gains

Acuity Mix

In addition to initial diagnosis, factors like comorbidity affect the reimbursement payment. EHC runs higher acuity in their MA book to make up for the discount on reimbursement vs FFS

KPI Benchmarking

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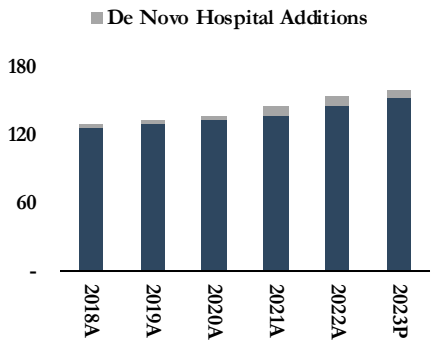
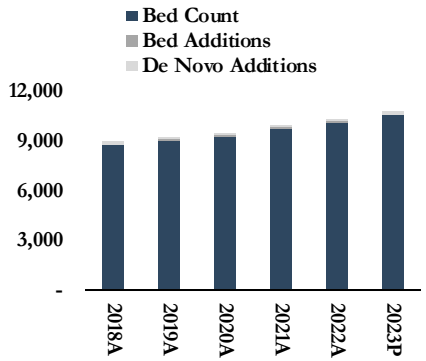
Breaking down the moving parts of strong growth

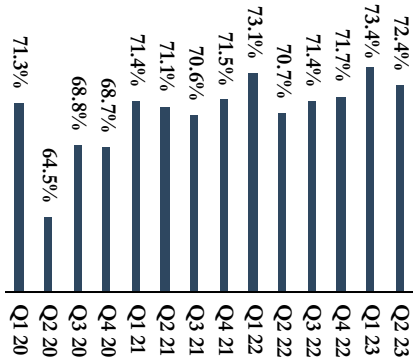
Growth Algorithm



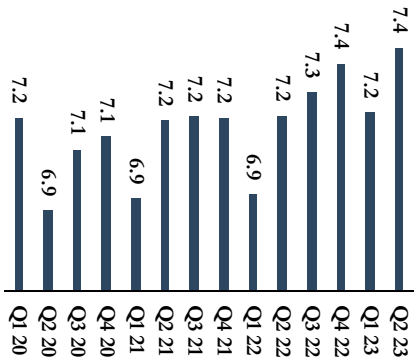
Facility Count



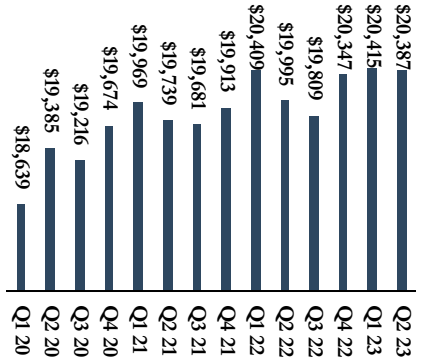
Bed Count



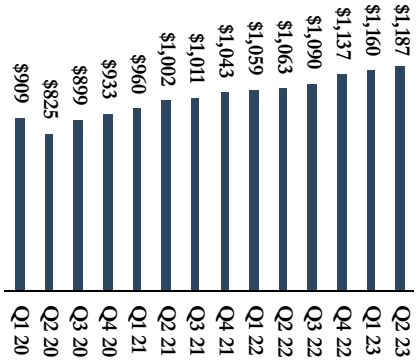
Bed Occupancy Rate



Discharges Per Occupied Bed



Revenue Per Discharge



Revenue

- **Occupancy**: EHC owns a vintage of semiprivate facilities, facilities built after FY12 are private hospitals with higher utilization at maturity. Expect increase in utilization as single bed hospitals grow as % of total fleet
- **Rev. Per Discharge:** Driven by negotiations with payors, Medicare FFS is largest payor making up ~65% of total reimbursement, MA is 2nd at ~15%
- **DPOB:** Primarily driven by acuity mix, average length of stay for an IRF is ~12.9 days vs ~34.5 days for SNFs, IRFs treat higher acuity patients than SNFs

Long Term Discharge Growth: EHC has a long runway for growth given the whitespace in the post-acute industry and strong value proposition, EHC targets 6-10 de novo hospital builds (~50 beds is the average among new builds) and 80-120 bed additions annually leading to a 6-8% discharge CAGR through FY27

Components of Revenue Growth

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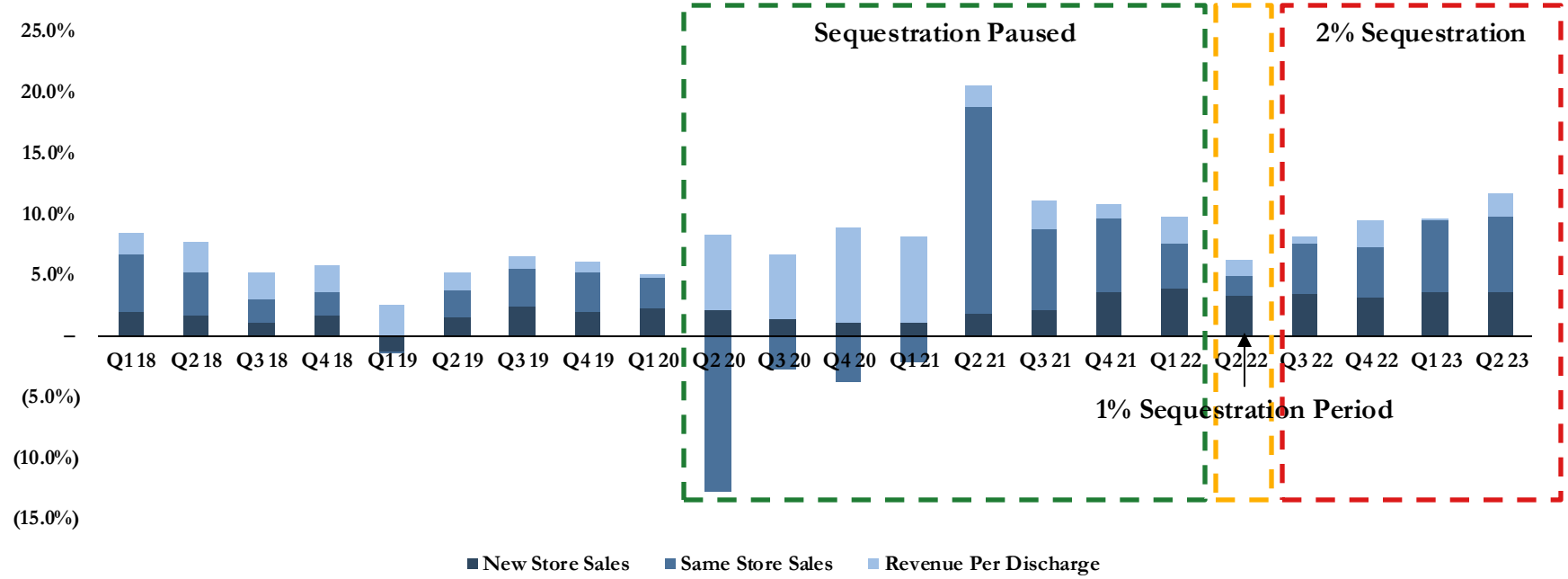
Understanding discharge growth is crucial for an investment in healthcare services

Price x Volume Breakdown



Revenue Per Discharge

Sequestration: 2% annual decline in payment for healthcare providers, was temporarily on pause during the PHE, resumed in April 2022. Full 2% sequestration now in effect, effectively reduces market basket updates for IRFs by 2%, investor focus on cadence of revenue per discharge growth vs SW&B per discharge

Acuity: During COVID, acuity mix shifted towards higher acuity procedures, led to an increase in revenue per discharge. Management believes acuity flows have completely normalized

Medicare FFS and MA Trends

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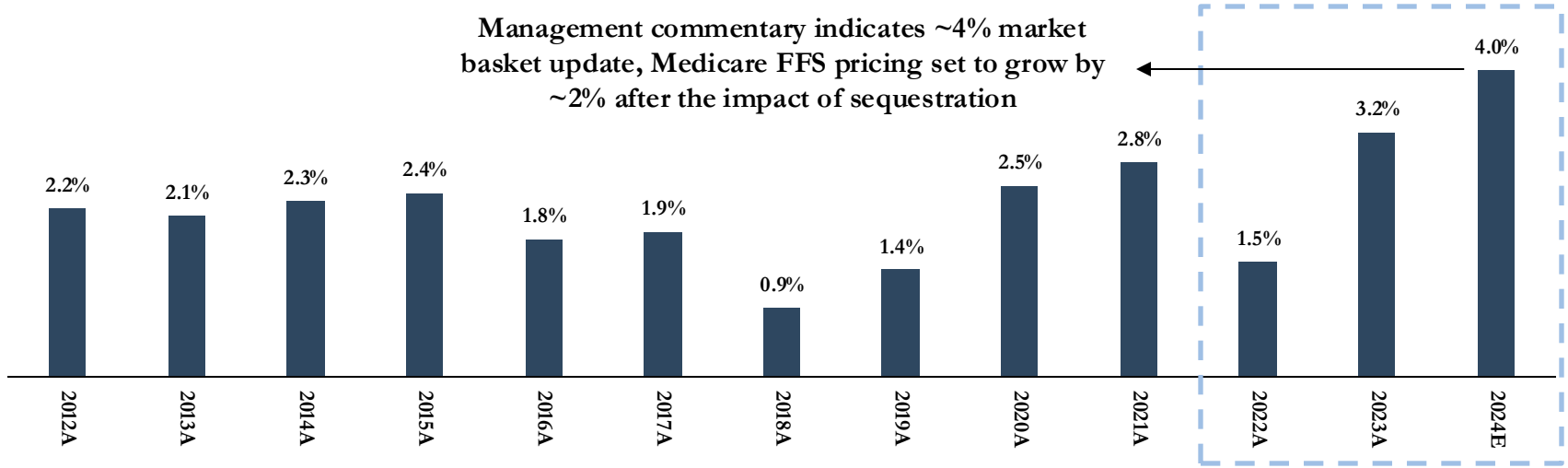
CMS determines rate updates for IRFs, comfort with reimbursement risk

IRF PPS Rate Update



Sequestration Impact

Resumption of sequestration in April of 2022, reduces PPS payments for IRFs by 2%. Market basket update is before the impact of sequestration, providers have experienced scrutiny from CMS and have had profits sequestered away

Reimbursement Risk

- Medicare FFS growth of 4% for the first three quarters of 2023, 2.5-3% in 4Q '23 (prior to the impact of sequestration). Revenue per discharge grew 2% Y/Y in Q2 '23, strong market basket adjustment offset by impact of sequestration
- Managed care pricing growth of 2-3% expected for the balance of the year
- Due to nature of rate determination (FFS pricing determined by CMS, MCOs have negotiating power on their bids due to their scale advantage), comfort with stroke of pen risk is important
- ~22% of IRFs are upside down on margins driven by labour pressures, expect market basket updates of 2%+ to cure industry wide margin pressure

Margin Trend

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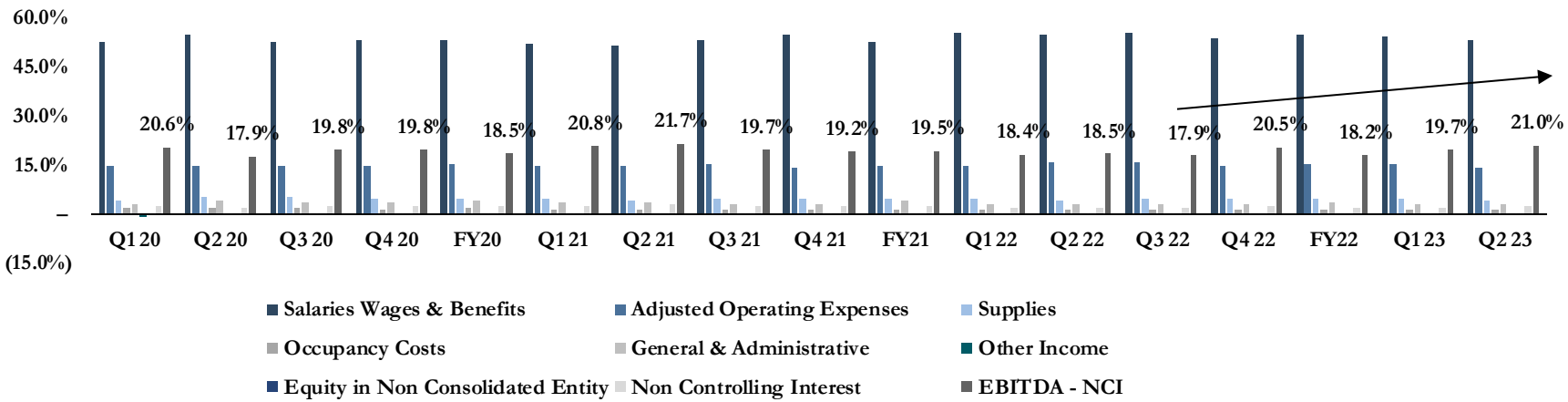
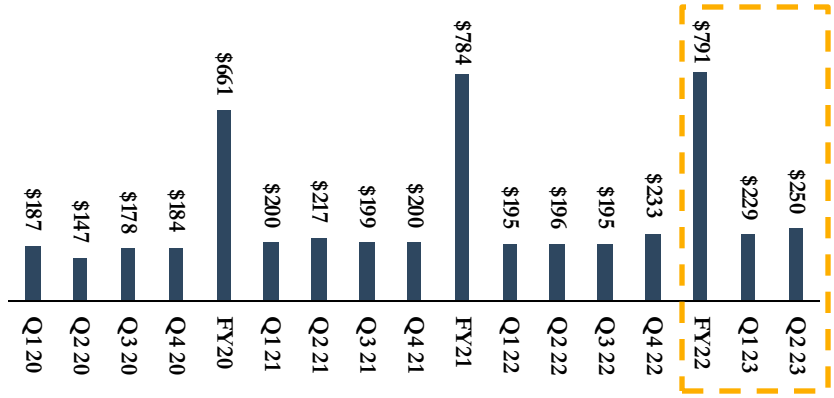
Salaries, wages and benefits inflation has applied downward pressure on margins

Input Cost Trends



EBITDA Trends



- **Margin Expansion:** Input cost inflation (primarily salaries and benefits) have applied downward pressures on EBITDA-NCI margins, as labour costs inflect downwards, encouraging trend of margin expansion to 20-21% range
- **EBITDA Growth**: EBITDA grew +27% in Q2 '23 and +22% for H1 '23. Strong growth trend driven by market share capture, industry tailwinds and margin expansion

Payor Dynamics

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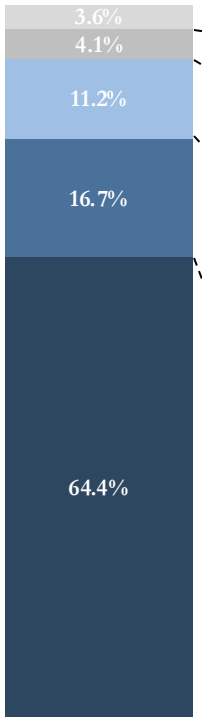
Double clicking on impact from Medicare Advantage share capture

Payor Mix



Other: Variety of methodologies

Medicaid: Varies by state

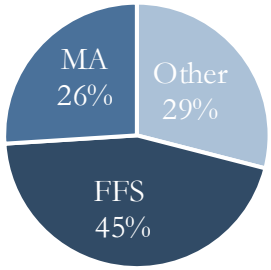
Managed Care: Per diem or CMG, negotiated rate, some are "tiered" for acuity/severity

Medicare Advantage: CMG or per diem, negotiated rate with payors, some are tiered for acuity/severity. ~85% of Medicare Advantage contracts have been negotiated away from per diem to CMG reimbursement. Combined with running higher acuity in the MA book, EHC sees MA reimbursed at ~5% discount to FFS

Medicare Fee For Service: Prospective Payment System ("PPS"), paid per discharge by Case Mix Group ("CMG")

- Medicare FFS
- Medicare Advantage
- Managed Care
- Medicaid
- Other

Commentary



EHC Stroke Payor Mix

Stroke as % of Discharges

"Unlike some of the difficulties that are faced in other post-acute sectors, we don't have a big reimbursement gap between Medicare Advantage and Medicare FFS. That number is running at less than 5% and that is – the payment differential is shrinking there based on 2 factors. One is, we have had a very concerted effort that has existed for almost a decade of trying to create – to transition MA contract from a per diem basis to a CMG basis, almost 90% of MA revenue is generated on a CMG basis (vs ~30% discount and 50% of contracts on CMG basis 10 years ago). The other factor is that we have historically run a higher acuity within our MA book of business than we have for fee-for-service. 1/3 of our MA book of business is stroke patients right now versus 14% for fee-for-service."

- Q2 '23 Earnings Call

Steady, Recession Resilient Grower

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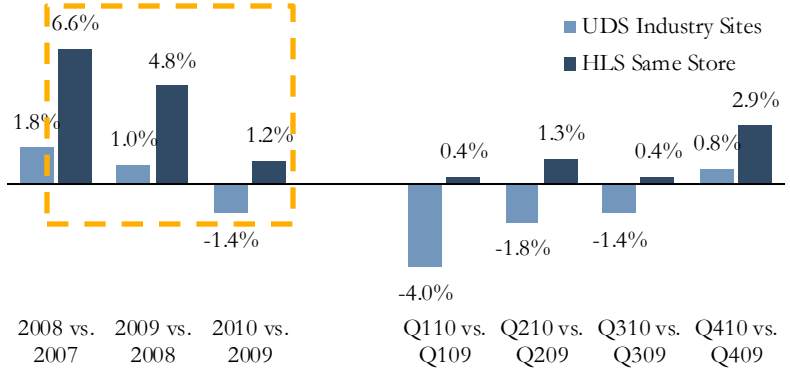
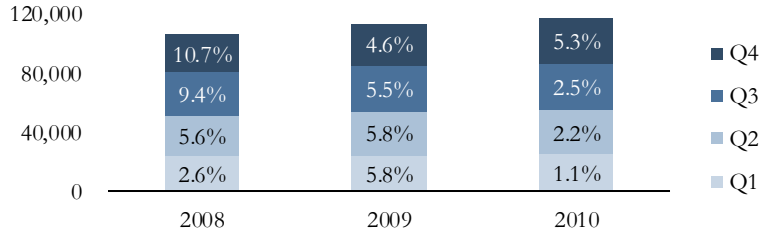
IRF discharge growth on SSS basis through GFC shows resilience

Resilient Discharge Growth



GFC Discharge Growth

Quarterly and Yearly Discharge Growth



Yearly Discharge Growth	7.0%	5.4%	2.8%

***Yearly Discharge Growth Not Same Store Basis**

Commentary

- EHC, like other healthcare investments, is largely recession resilient. As evidenced by strong discharge growth (~6.6% in FY08 and ~4.8% in FY09), a recession has a negligible impact on IRF demand, positive demographic trends and value proposition remain the largest drivers of discharge growth
- **Recession Case – Labour Impact:** EHC significantly benefits on the cost side in the case of a recession. Recent surge in SW&B and contract labour hiring should normalize to pre-pandemic levels (SW&B inflation of ~3% annually, contract labour at 0.9% of total FTEs), resulting in margins to inflect upwards (currently all fixed cost leverage is neutralized by pressures on the variable side)

> "Given the nature of our patients and the fact that it's the senior population, we just have not seen an impact historically from recessions. If anything, it's been recession-proof it doesn't impact our business a great deal."
>
> - Q1 '20 Earnings Call

Attractive Facility IRRs

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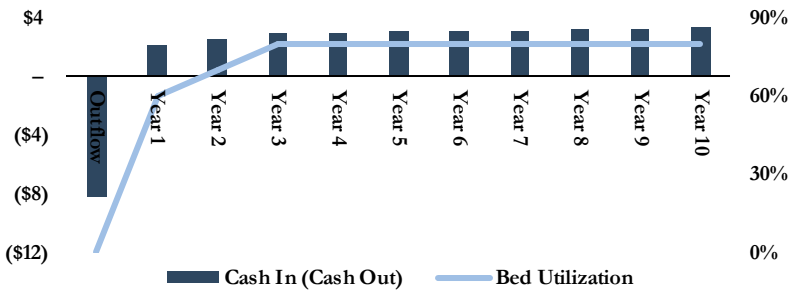
19% ROIC for facilities opened since 2009; blend of de novo and bed addition capital deployments

Bed Additions Math

Bed Additions IRR Analysis *Figures in Millions Unless Otherwise Stated*	
Cost Per Bed Addition	$415K
Revenue Per Discharge	$20,415
Average Bed Adds Per Facility	20
EBITDA Contribution Margin	30%
Revenue Per Discharge Growth	2%
10-Year Bed Additions IRR	**30%**



- Unlevered IRRs on bed additions are very attractive (in-line with management commentary of ~35-40%)
- EHC makes bed additions when private facilities reach ~85% occupancy (~80% for semi-private facilities); certain states have CON requirements for bed adds with exceptions above certain utilization thresholds

De Novo Facility Math

De Novo Facility IRR *Figures in Millions Unless Otherwise Stated*	
Cost Per Facility	$48M
Revenue Per Discharge	$20,415
Average Bed Adds Per Facility	20
Bed Addition Cost	$8M
EBITDA Contribution Margin	–
Revenue Per Discharge Growth	3%
Supplies % of Sales	5%
Operating Expenses % of Sales	15%
SW&B Annual Growth Rate	3%
15-Year Unlevered IRR	**12%**

De Novo ROIC

- 2009 to 2020 de novo cohort (consists of 31 de novos, 16/31 have had bed additions post-opening) have generated an aggregate ROIC of ~19.2%
- 2021 and 2022 cohort (16 additional de novo builds) generated an LTM ROIC of 11%, exceeding WACC in less than 3 years post-opening
- Investor day commentary reinstates strong returns on capital

Labour Costs Inflection

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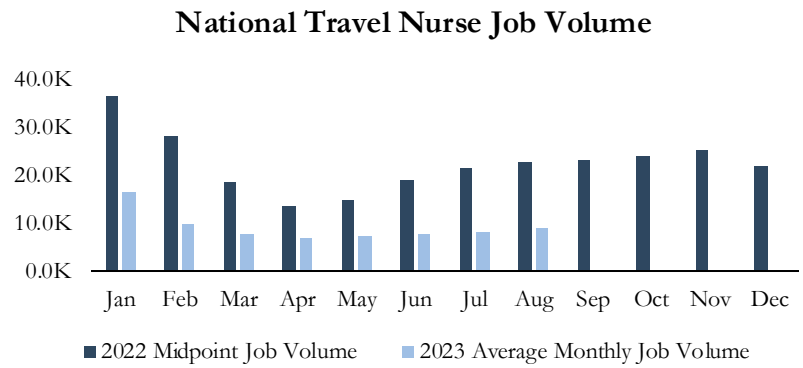
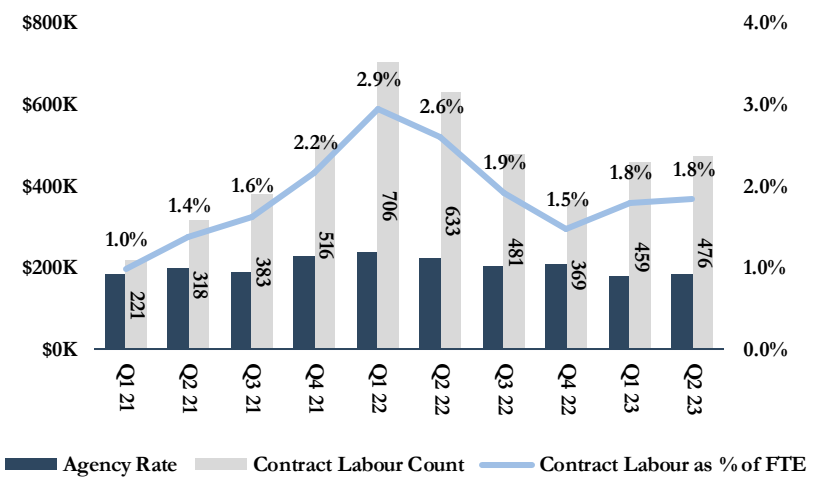
Contract labour utilization and rate inflecting downwards, upside on ST EBITDA margins

Industry Wide Contract Labour Utilization



Travel nurse volumes inflecting downwards, expect easing of contract labour utilization and rates

EHC Utilization and Agency Rate



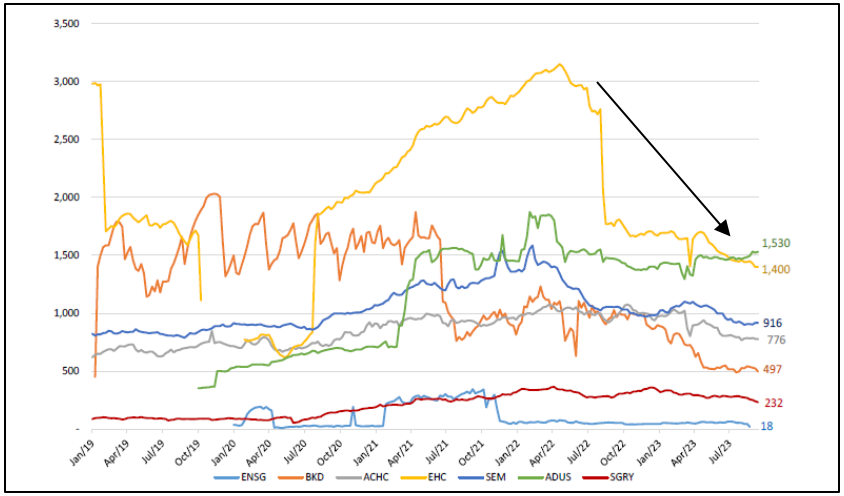
- Modelled contract labour as % of total FTEs at 1.6% for 2H23, decreasing to 1.1% LT vs 0.9% pre-Covid average
- "Is 0.9% realistic anytime soon? We don't have the answer to that yet. Our focus is to make incremental progress driving down that 1.8% figure. We expect contract labour FTEs as a % of total FTEs to remain relatively flat for 2H23"
- SW&B expense trend at ~55% of revenue vs pre-COVID average of ~51% driven by labour inflation and contract labour utilization

Labour Tracker

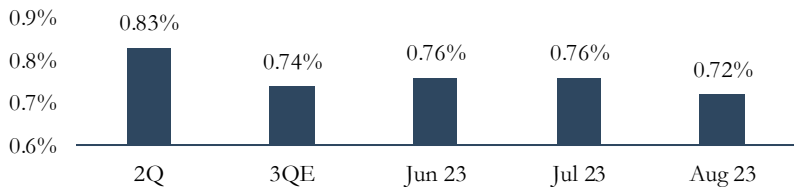
Encompass Health

Sell-side furnished alt data indicates that post-acute labour woes have improved

Post-Acute Nursing Job Postings



Nurse Openings as % of Discharges



Job Postings by Site of Care

	W/W growth	M/M growth	YTD	YOY
UHS	(1.5%)	(4.0%)	(8.7%)	(17.8%)
THC	(1.4%)	(8.9%)	(52.2%)	(49.4%)
HCA	(2.5%)	(7.1%)	(18.5%)	(20.3%)
CYH	(0.4%)	(2.7%)	(34.0%)	(38.0%)
Acute Care	**(1.8%)**	**(6.3%)**	**(28.2%)**	**(29.7%)**
EHAB	(1.5%)	(5.3%)	(12.9%)	(14.7%)
AMED	(0.5%)	(4.3%)	(12.7%)	(33.2%)
AVAH	0.1%	0.7%	11.4%	12.2%
Home Health	**(0.5%)**	**(2.3%)**	**(2.8%)**	**(11.1%)**
ENSG				(15.4%)
BKD	(5.0%)	(8.3%)	(44.6%)	(48.6%)
ACHC	(0.4%)	(1.1%)	(20.1%)	(20.6%)
EHC	0.1%	(3.0%)	(16.2%)	(22.2%)
SEM	(0.1%)	1.1%	(7.5%)	(13.3%)
ADUS	0.5%	3.1%	10.7%	4.5%
SGRY	(2.9%)	(13.4%)	(31.4%)	(21.1%)
Post-acute (non-HH)	**0.3%**	**(1.4%)**	**(14.3%)**	**(18.4%)**

Alt Data Analysis

Labour trackers indicate that EHC has turned the page on its staffing woes, "fast-money" investors pay close attention to alt data. EHC is an imminent margin expansion story, reasonable to exit position to long-only investors after thesis plays out

Contract Labour Upside

Encompass Health

Margin expansion runway as utilization of contract labour normalizes

Contract labour Upside	Q1	Q2	Q3	Q4	2022A
Revenue	**$1,059**	**$1,063**	**$1,090**	**$1,137**	**$4,349**
Salaries, Wages and Benefits	**587**	**586**	**606**	**614**	**2,393**
Gross Profit	**$472**	**$477**	**$484**	**$523**	**$1,955**
Gross Profit Margin	44.5%	44.9%	44.4%	46.0%	45.0%
Contract Labour FTE Count	706	633	481	369	547
Total Employee Count	24,019	24,282	25,061	25,146	24,627
Contract Labour as % of Total FTEs	2.9%	2.6%	1.9%	1.5%	2.2%
Contract Labour Rate (Annualized)	$240K	$223K	$205K	$211K	$223K
Projected Contract Labour as % of Total FTEs	1.2%	1.2%	1.2%	1.2%	1.2%
Implied Contract Labour Count	288	291	301	302	296
Regular FTE Rate (Annualized)	$96K	$96K	$96K	$96K	$96K
EBITDA Contribution	**$15**	**$11**	**$5**	**$2**	**$32**
2022A EBITDA	$195	$196	$195	$233	$874
2022A EBITDA Margin	18.4%	18.5%	17.9%	20.5%	20.1%
EBITDA Upside	**7.7%**	**5.5%**	**2.5%**	**0.8%**	**3.6%**
PF EBITDA Margin	**19.8%**	**19.5%**	**18.4%**	**20.6%**	**20.8%**
Margin Expansion	**142 bps**	**102 bps**	**45 bps**	**17 bps**	**73 bps**

Implied EBITDA Margin		FTE Rate				
		$193K	**$203K**	**$213K**	**$223K**	**$233K**
Normalized Contract Labour % of Total	**1.0%**	20.8%	20.8%	20.9%	21.0%	21.0%
	1.1%	20.7%	20.8%	20.8%	20.9%	21.0%
	1.2%	20.6%	20.7%	20.8%	**20.8%**	20.9%
	1.3%	20.6%	20.6%	20.7%	20.8%	20.8%
	1.4%	20.5%	20.6%	20.6%	20.7%	20.7%

Implied Margin Expansion		FTE Rate				
		$193K	**$203K**	**$213K**	**$223K**	**$233K**
Normalized Contract Labour % of Total	**1.0%**	67 bps	74 bps	81 bps	87 bps	94 bps
	1.1%	61 bps	68 bps	74 bps	80 bps	87 bps
	1.2%	56 bps	62 bps	67 bps	**73 bps**	79 bps
	1.3%	50 bps	56 bps	61 bps	66 bps	71 bps
	1.4%	45 bps	50 bps	54 bps	59 bps	63 bps

Contract Labour

Contract labour usage increased during the pandemic due to nurses being more willing to work as contractors, normalized levels of ~0.9% of total FTEs vs ~2.2% for FY22, **~1.2% modelled as normalized baseline**

Margin Expansion

EHC's margin woes from SW&B inflation and contract labour hiring have offset any realized leverage on the fixed cost side. As EHC's labour pains ease, we see a glide path to **low-20% EBITDA – NCI margin** (evidenced by 2Q23 beat on street margin estimates)

***Analysis isolates for contract labour normalization, fully normalized FY22 not reflected**

Home Health Transfer Exemption

Encompass Health

Washington Analysis report on IRF home health transfer rule sent investors into a frenzy

Situation Update

December 2021 – Office of Inspector General publishes report on IRF Home Health Transfer Rule → **Early 2022 – CMS seeks out comments from IRF in tandem with FY23 PPS rate update** → **July 2022 – FY23 PPS ruling ignores home health transfer exemption**

- IRF Transfer Rule: CMS established a transfer payment based on a per diem amount that would reduce the payment for each case-mix group for which the discharge occurred earlier than the average length of stay. This policy applies to 1) another IRF 2) inpatient hospital 3) nursing home 4) LTACH. However, the policy does not apply to early IRF discharges to home healthcare. OIG's report on IRF claims showed that the CMS could have saved ~$1B in FY17 and FY18 combined if they abolished the home health transfer exemption

- Home health transfers from IRFs are continuation of care patterns, not substitution, CMS isn't "double counting" discharges with the transfer exemption
- While the proposed rule would decrease payments for IRFs in transfer cases where LOS is lower than average LOS for the case-mix group, the analysis doesn't consider the excess costs for IRFs for cases that extend beyond the average LOS and continue to be reimbursed on CMG rather than a per diem basis

- Per CMS ruling on IRF transfers, IRFs will not receive full CMG payments for transfer that are categorized as "substitution of care". IRF acuity mix and 60% of discharges mandated to be in the CMS-13 category means IRF cases are highly specialised, and patients continue to need care at home. CMS hasn't yet released a follow-up ruling on the exemption, further reform might be in the cards for PPS update for FY24 or FY25

Investor Day Update: Management noted that EHC has been very active in Washington D.C. with regulators (CMS) and there are no currently imminent signs of any forthcoming legislation on this issue

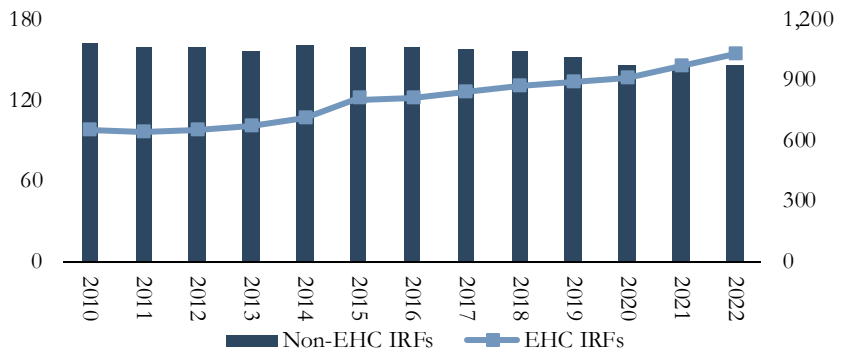
Favourable Supply-Demand Dynamic

Encompass Health

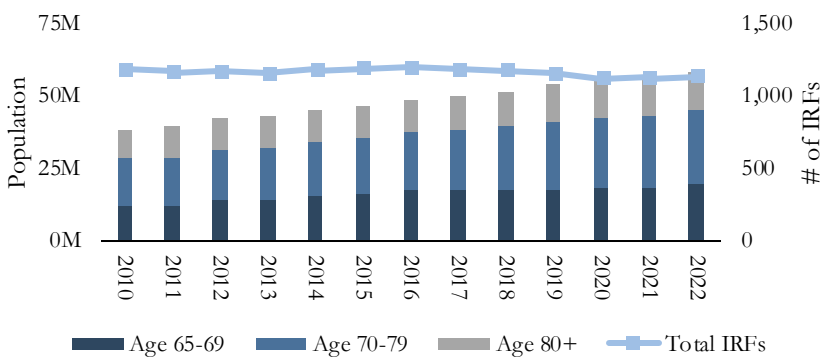
Industry facility count is flat over L10Y despite strong demographic growth

Favourable Facility Dynamics

Supply of IRFs – EHC vs. Non-EHC



Demand Backdrop – Core Population



Commentary

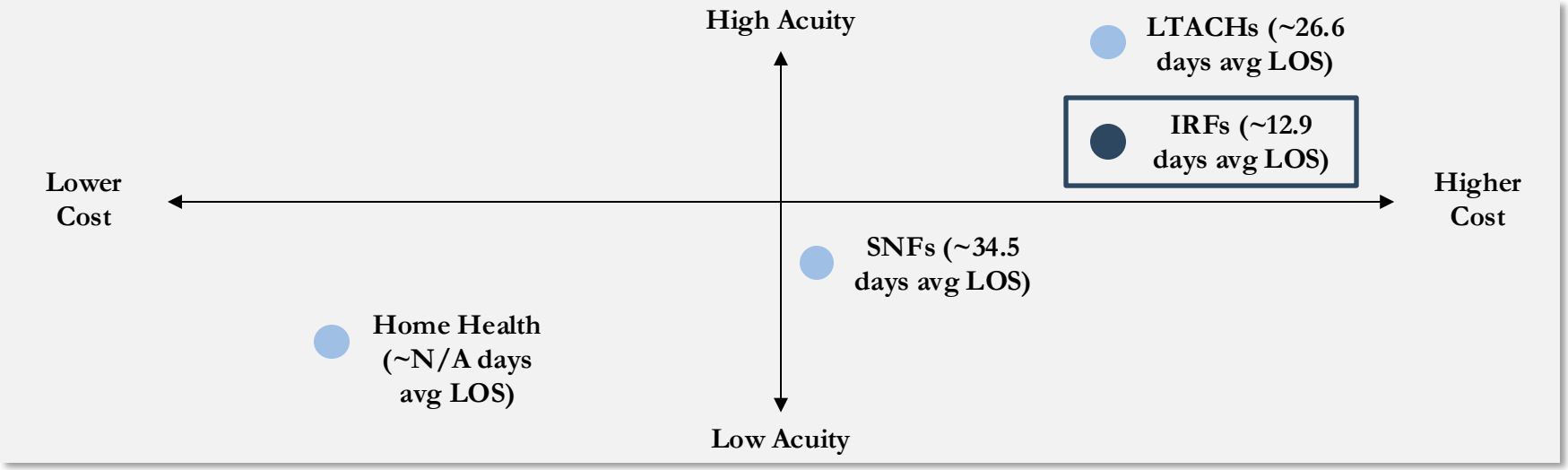
- **Demand:** Post-acute as an industry significantly benefits from strong demographic trends; core population cohort growing at a 5% CAGR through the balance of the decade
- Demand for IRFs driven by increased payer understanding of their strong value proposition, quality of outcomes (67.6% discharge to community rates for IRFs vs 43.5% for SNFs, EHCs fleet of IRF's have a 16.1% 30-day readmission rate vs 17.6% for national average)
- ~10% of IRFs receive 53% of the outlier payments, none of the 10% are EHC IRFs. EHC receives ~3.7% of total Medicare outlier payments despite treating ~31% of Medicare discharges. Medicare pays ~$871 less per discharge on average than other providers
- **Supply:** Over the last decade, the number of IRFs in the US has stayed roughly flat while EHC has doubled facility count
- Supply is constrained by:
  - Highly regulated industry with "CMS-13" compliancy
  - Significant capital investment (~$50M for a 50-60 bed facility)
  - Existing players have established payor and referral source relationships
  - Certificate of Need required to build facilities and undertake bed additions in certain states
- **~13% of acute care patients with an eligible "CMS-13" diagnosis are eventually admitted into an IRF, significant runway for growth**

Competitive Landscape

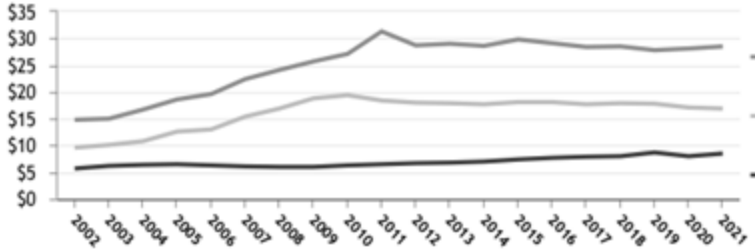
Encompass Health

Lay of the land in post-acute care

Benchmarking Post-Acute Care Options



Medicare Spending on Post-Acute



Post-Acute setting	2002 Spending	2021 Spending
Skilled nursing facilities	$14.8	$28.5
Home health agencies*	$9.6	$16.9
Inpatient rehabilitation hospitals	$5.7	$8.5

$ in billions

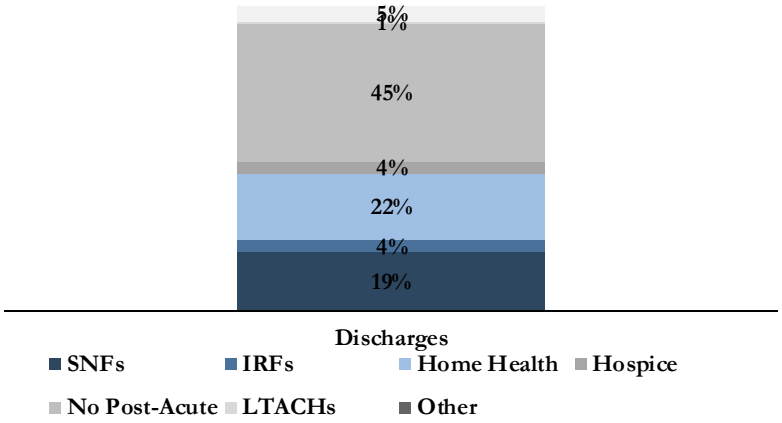
Medicare spending on IRFs is going to see a significant growth tailwind. Growth driven by strong demographic trends and site of care shift

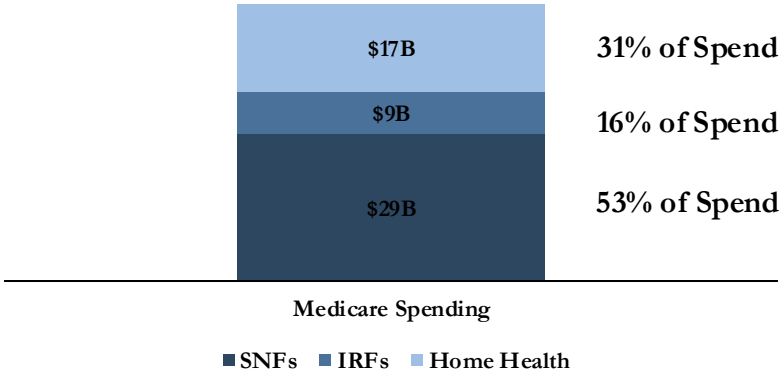
Post-Acute Discharge Breakdown

Encompass Health

IRFs are an attractive way to play strong demand pipeline

Destination by Discharges and Medicare Spend





Commentary

- **Home Health:** Medicare reimbursement pressures means the sector is a sub-optimal way to gain exposure to strong demographic trends. Public players have traded at 20x+ multiples previously due to strong demand from an aging population and a shift to care at home
- **SNFs:** Labour woes have made the industry extremely unattractive, much more adversely impacted by input cost inflation, ~25% of providers are expected to file for bankruptcy over the next 12 months. Per Gibbins Advisors, ~54% of healthcare bankruptcies over the past 18 months were in the senior living and skilled nursing sector. Per the company, more than 600 nursing homes have closed over the past 6 years

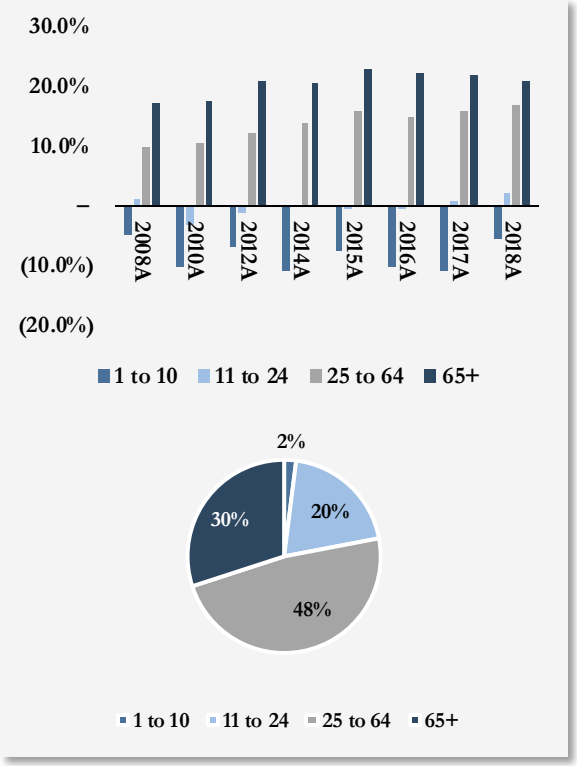
~13% of "CMS-13" eligible discharges from acute care facilities end up in IRFs. Supply growth of post-acute care remains anemic (home-health agencies face reimbursement pressures from CMS, SNFs deal with labour shortages, freestanding IRF count is flat over L20Y). During the pandemic, there was a temporary shift of MA volumes to IRFs, given the continued mismatch as we exit the pandemic, expect some shift of volumes to be sticky

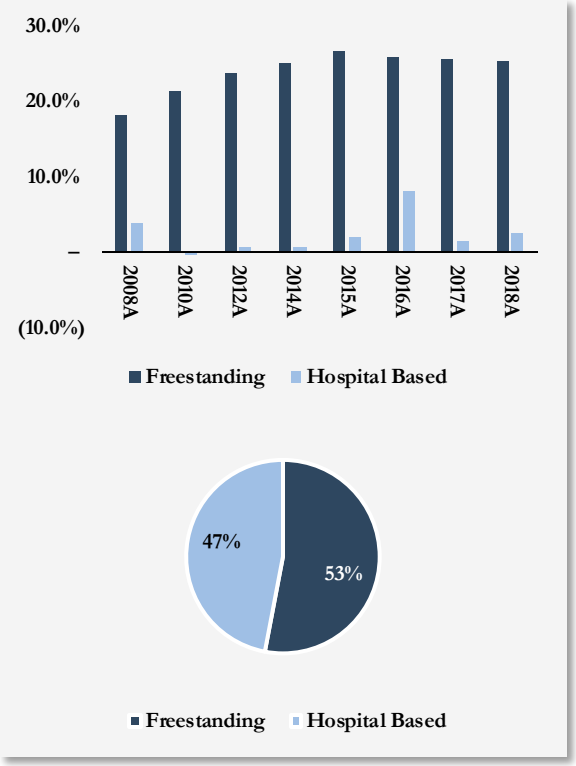
Platform Quality

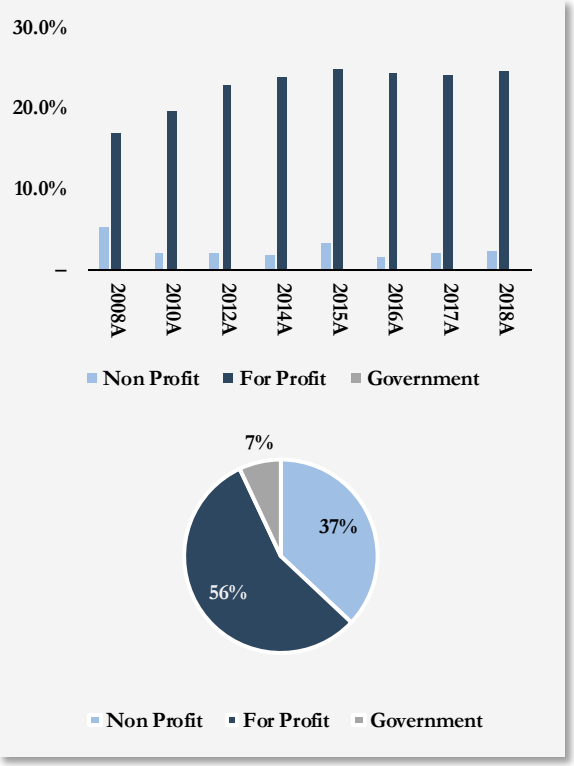
Encompass Health

IRFs benefit from scale and specialization, 4-wall EBITDA margins of ~25% for scaled facilities

Margin Profile & Discharge Share by Facility Type







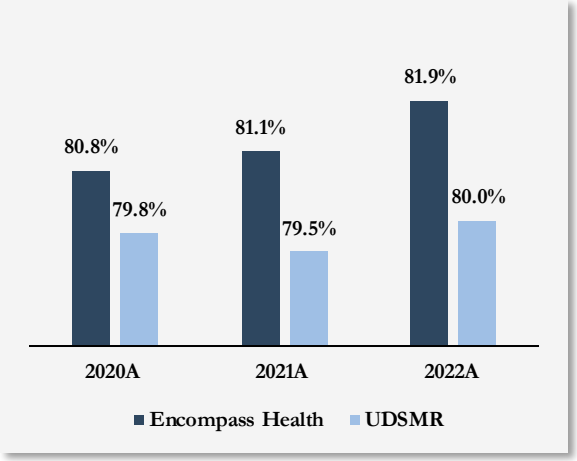
~22% of IRFs are upside down on margins, IRFs benefit significantly from scale, 4-wall EBITDA margins for EHC's 50+ bed facilities are 25%+, hospital-based facilities lack specialization to operate successfully

Value Proposition

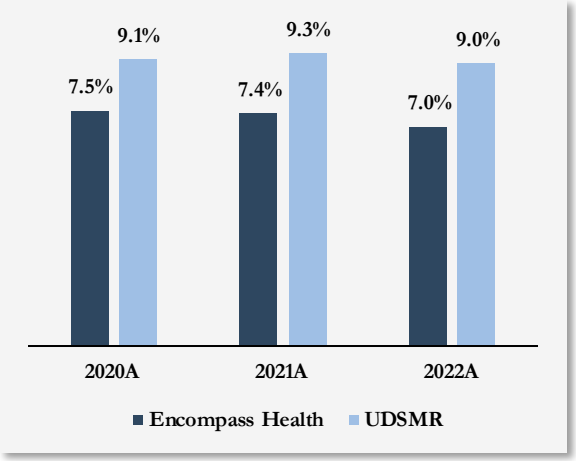
Encompass Health

Strong value proposition vs competitor IRFs and SNFs leads to share gains

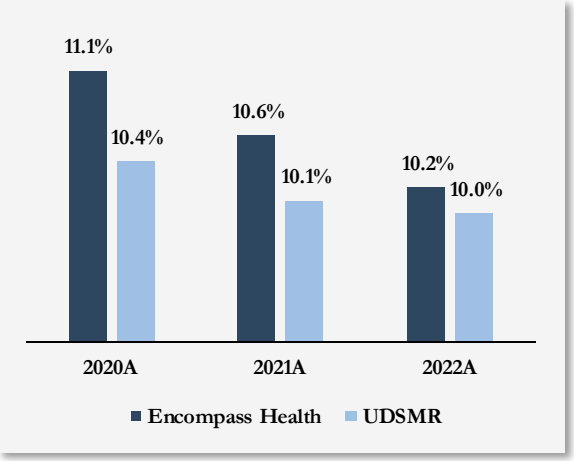
Strong Value Proposition for Payors



Discharge to Community (Higher is Better)



Discharge to SNFs (Lower is Better)



Discharge to Acute Care (Lower is Better)

- Encompass generates better outcome for patients due to their expertise and specialization in IRF management. IRF hospitals have a unique case mix with a higher incidence of stroke, neurological disorders etc. Acute care hospitals with IRF wings lack the specialization to efficiently operate IRFs, average fleet bed count for hospital units is smaller
- EHC also generates minimal outlier payments for payors (outlier payments defined as cases where an extremely adverse outcome leads to an outlier adjustment to the reimbursement amount, less outlier payments lead to more predictable loss ratios for payors), 10% of IRFs receive 53% of the outlier payments, none of the 10% are EHC IRFs, EHC receives 3.7% of IRF outlier payments despite treating 30%+ of Medicare discharges

EHC IRFs lead to better outcomes vs UDSMR IRFs and SNFs (~76% discharge to community rates for IRFs vs ~41% for SNFs), lower outlier payments, strong value proposition for payors

Cost of Care Benchmarking

Encompass Health

EHC discharges cost payors less than free-standing IRFs and hospital units

Cost of Care Benchmarking

*Analysis assumes a comparable case mix

	Number of IRFs	Avg Beds Per IRF	Avg FFS Discharges	Cost Per Discharge	Payment Per Discharge
Encompass Health	152	67	833	$16,627	$24,307
Free-Standing	196	57	540	$22,351	$26,378
Hospital Units	779	24	196	$26,240	$27,027
Total	1127	36	340	$22,010	$25,952

EHC's fleet of IRFs have an average of ~67 beds (bed count is exponentially accretive to margins), allows EHC to manage cost per discharge, payments for payors are significantly lower than an average IRF

Market Share Capture

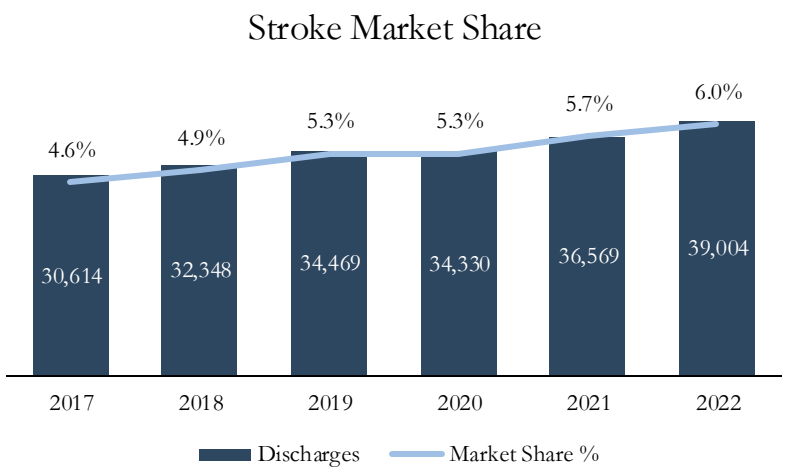
Encompass Health

Exceptional platform quality supports above market growth

Commentary

- EHC IRFs have a larger fleet size, IRFs with 50+ beds benefit from economies of scale and generate 20%+ 4-wall EBITDA margins vs breakeven for IRFs with <25 beds. Advantaged cost structure drives market share capture and churn among the less scaled competitors
- Strong value proposition for players (80%+ discharge to community rate) and lowest outlier payments (EHC IRFs receive ~3% of outlier payments despite treating ~30% of all Medicare discharges)
- **COVID Strategy**: Despite facing labour woes during the pandemic, EHC aggressively captured discharges. Taking on additional volumes (especially Medicare Advantage) showed payors the strong value proposition of EHC IRFs
- "We think our value proposition with our outcomes continue to help us to grow volumes…we like getting that additional volume gain and we like that our value proposition beyond stroke where it's been evident for a long period of time and complex neurological disorders is now resonating with MA plans …our value proposition continues to resonate, and we are experiencing market share gains"
- Higher acuity procedures like stroke and other neurological conditions are shifting to IRFs, EHC continues to gain share from SNFs and other post-acute settings, strong value proposition resonates with MA payors
- Net-net only 13.5% of CMS-13 eligible discharges end up in IRFs, we see significant white space for share capture and above market growth due to EHC's operational chops and strong value proposition

EHC Share Capture



Long Term Guidance

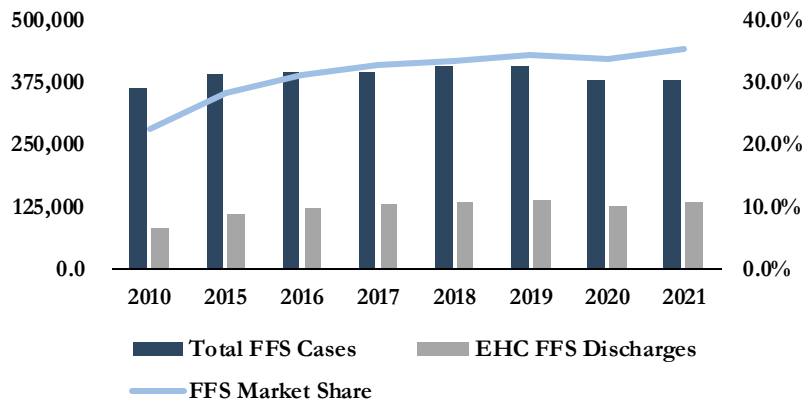
Management targets 6-8% long term discharge CAGR, EHC specialization in rehabilitation services, strong industry tailwind and share gain potential means EHC can comfortably grow at the higher end of the target range MA payors resonate with EHC value proposition and continue to move higher acuity volumes to IRFs, higher acuity is accretive to IRF pricing

Share Gains

Encompass Health

EHC has captured share within the IRF industry, IRFs are more profitable

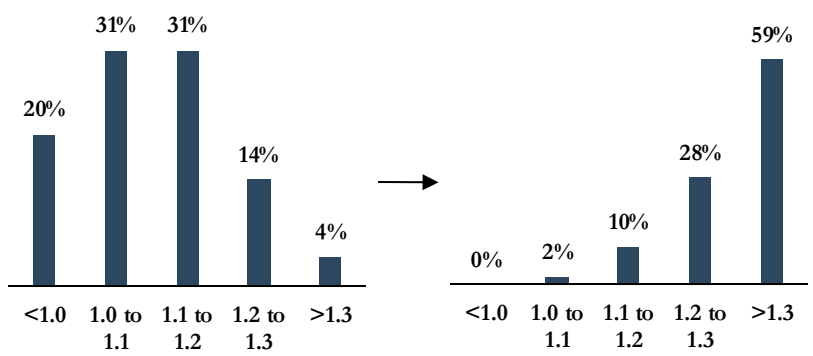
EHC Captures IRF Share



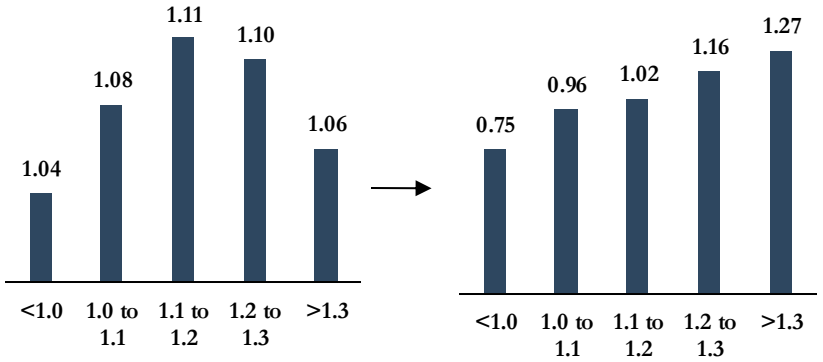
Total Medicare FFS Spend on IRFs

- EHC IRFs have steadily captured share within Medicare FFS (increase from ~22% of discharges in 2010 to ~36% of discharges in 2021), strong value proposition has resonated with Medicare payors
- Further, a shift in site of care is apparent as higher acuity cases make up a larger share of IRF discharges, highest acuity cases are most profitable

IRFs Acuity Increase Leads to Higher Profitability



2007 vs 2021 Case Mix Indicates Acuity Increase



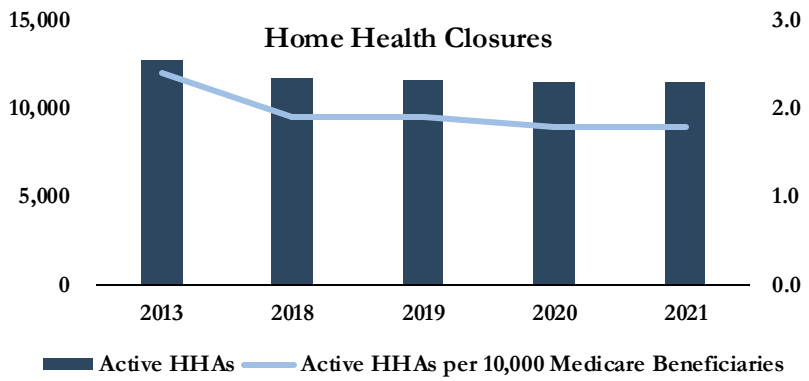
2007 vs 2021 Profit to Cost Ratio Growth

Expressing Thematic View

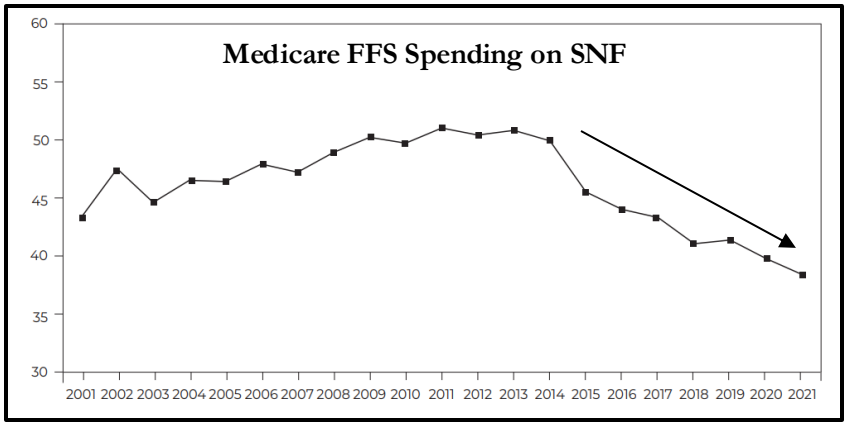
Encompass Health

Value proposition resonates with payors, share shift within post-acute care

Home Health Headwinds

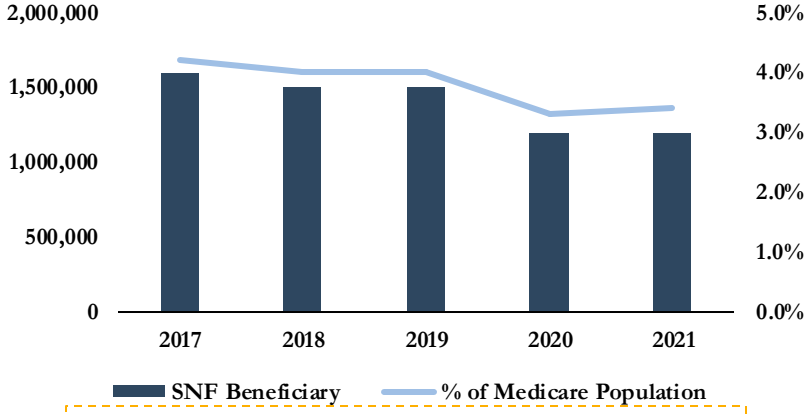


SNFs Lose Market Share



HH Pricing Headwinds

- Net rate update for HH in CY24 expected to be -2.2%, comprised of a +2.7% headline increase (+3.0% market basket increase less a 0.3% productivity adjustment), and a -5.1% behavioral adjustment and a 0.2% increase due to the update to the fixed-dollar loss ratio
- CMS belief that the agency overpaid home health providers by ~$1B a year over the two-year period between 2020 & 2021



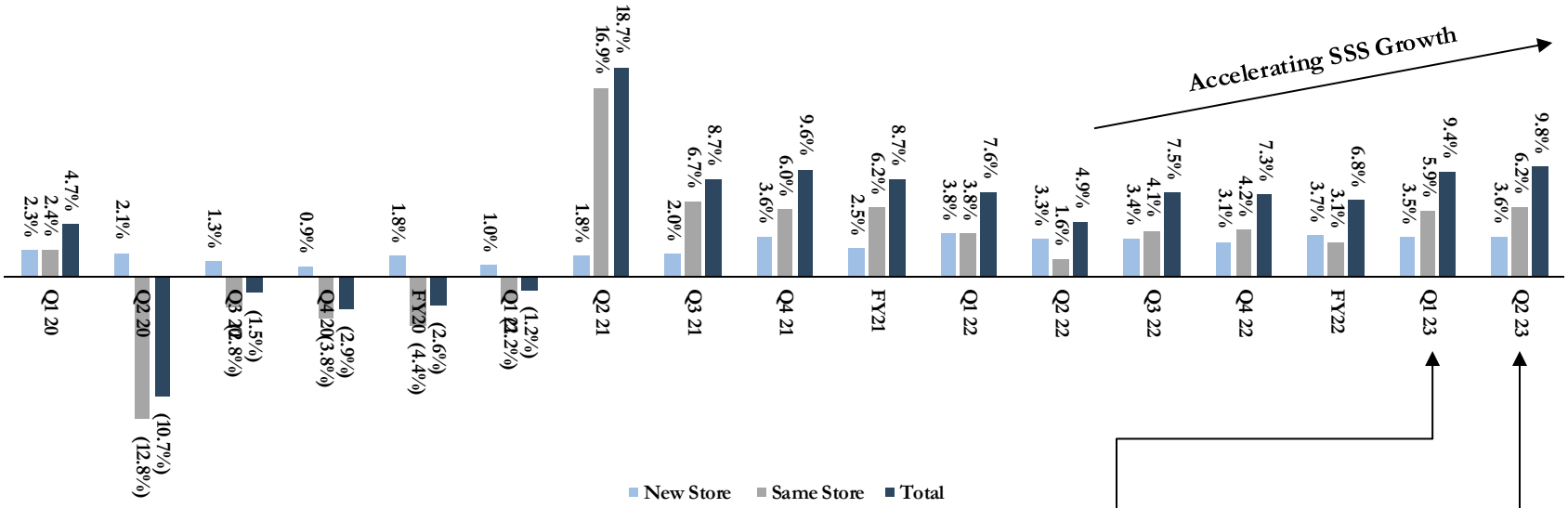
SNF Beneficiaries Decrease as % of Population

Growth Algorithm

Encompass Health

6-8% discharge CAGR driven by bed additions, de novo builds and organic SSS growth

Benchmarking Post-Acute Care Options



Exit Rate

- 9%+ YoY growth in H1 23, strong utilization trends across Medicare Advantage and FFS. Management commentary indicates normalized flows and case mix
- FY23 volumes driven by strong SSS performance (reacceleration for L3Q), 31 bed adds and 2 new de novo builds

154 total bed adds (149 across de novos, 5 added to existing facilities), strong trends across FFS (+9% YoY) and Medicare Advantage (+20% YoY, 17% SS)

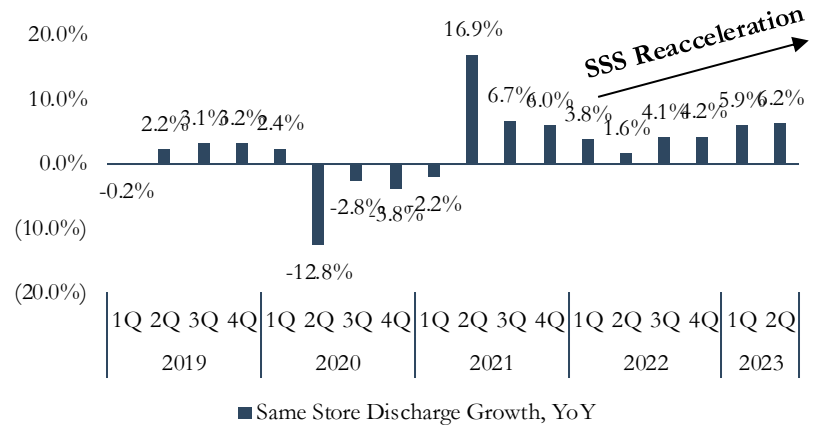
120 total bed adds (110 across de novos, 10 added to existing facilities), strong trends across FFS (+9% YoY) and Medicare Advantage (+20% YoY)

Strong Discharge Trend

Encompass Health

MCO commentary read through suggests strong utilization and SSS trends through FY24

SSS Growth Reacceleration



Utilization Drivers

Demographic Growth: Core population CAGR of ~4% for N10Y is a tailwind for post-acute providers

Post-Pandemic Labour Policy: EHC hired contract labour to meet demand during COVID, downward pressure on margins mitigated by strong volumes

Share Capture: Strong value proposition for MA payors has led to share gains from competing IRFs and other post-acute providers, expect partial shift in site of care to be sticky

New Facility Builds: 6-10 IRF planned annually, bed additions drive discharge count

MCO Commentary

"Our 2024 MA bid did contemplate a degree of higher utilization…our MLR increased 350 bps Y/Y reflecting higher than expected Medicare Advantage utilization…we also recognized higher utilization levels in Q1."
- CVS Q2 Earnings Call

"In mid-June 2023, UNH indicated that MLR would be in the upper half of its guidance range primarily due to elevated Medicare Advantage activity. UNH's 2024 Medicare Advantage bids assume elevated utilization will persist next year…"
- JPM on UNH

"Humana reiterated that utilization trends have stabilized at a higher level than June and they expect trends will continue into 2024. Management comments suggest that HUM MLR will be running higher in FY24"
- Bernstein on HUM

View on Upcoming Quarter and FY23

Encompass Health

IRF utilization trend and bed additions suggest conservative FY23 guidance

Strong Discharge Trend in H1

	FY2020	FY2021	FY2022	31-Mar-23 Q1	30-Jun-23 Q2	30-Sep-23 Q3E	31-Dec-23 Q4E	FY2023
Total Facility Count								
Starting Bed Count	9,249	9,505	9,924	10,356	10,510	10,611	10,666	10,356
Net Bed Adds	256	419	432	154	101	55	55	365
Bed Additions	-	-	-	5	(9)	16	16	27
De Novo Openings	-	-	-	149	110	40	40	338
End Bed Count	9,505	9,924	10,356	10,510	10,611	10,666	10,721	10,721
Bed Count Growth Y/Y		4.4%	4.4%	4.8%	4.0%	3.0%	3.5%	3.5%
Bed Occupancy Rate				73.4%	72.4%	72.5%	72.5%	
Occupied Beds				7,714	7,682	7,733	7,773	
Discharge Per Occupied Bed	28.3	28.5	28.8	7.2	7.4	7.4	7.4	29.5
Y/Y Growth		0.7%	1.0%	4.0%	3.2%	2.1%	0.7%	2.5%
Total Discharges	**181,897**	**197,639**	**211,116**	**55,557**	**57,011**	**57,388**	**57,684**	**227,641**
Total Discharge Growth		8.7%	6.8%	9.4%	9.8%	6.8%	5.5%	7.8%
Revenue Per Discharge				**$20,415**	**$20,387**	**$20,205**	**$20,653**	
Y/Y Growth				0.0%	2.0%	2.0%	1.5%	
Inpatient Revenue	**$3,496**	**$3,918**	**$4,252**	**$1,134**	**$1,162**	**$1,160**	**$1,191**	**$4,647**
Y/Y Growth		12.1%	8.5%	9.5%	12.0%	8.9%	7.0%	9.3%
Outpatient Visits	186,257	161,070	138,644	31,852	30,752	35,035	33,801	131,440
Y/Y Growth		(13.5%)	(13.9%)	(9.6%)	(14.4%)	2.0%	2.0%	(5.2%)
Revenue Per Outpatient Visit				$823	$806	$732	$741	
Y/Y Growth				25.4%	17.3%	1.0%	1.0%	
Outpatient Revenue	**$70**	**$97**	**$97**	**$26**	**$25**	**$26**	**$25**	**$102**
Total Revenue	**$3,566**	**$4,015**	**$4,349**	**$1,160**	**$1,187**	**$1,185**	**$1,216**	**$4,749**

Modelled 365 Net Bed Additions in FY23: Bed forecasts driven by de novo additions (5 opened YTD, 1 additional opening each in Q3 and Q4, 31 planned bed additions for the balance of FY23)

Revenue Per Discharge Growth: 4% IRF PPS adjustment translates to 2% net growth in Q3 after sequestration and 1.5% in Q4. Management commentary suggests case mix index is normalized post-pandemic

FY23 guidance of $4,750 to $4,810, base case model forecast of $4,747 (+9% Y/Y). Expect actual print to be on higher end of the range given positive setup

View on Upcoming Quarter and FY23

Encompass Health

SW&B improvement combined with conservative guidance greenlights FY23 beat

Conservative Guidance After Huge Beat

	31-Mar-23 Q1	30-Jun-23 Q2	30-Sep-23 Q3E	31-Dec-23 Q4E	FY2023
Total Revenue	**$1,160**	**$1,187**	**$1,185**	**$1,216**	**$4,749**
Salaries, Wages and Benefits	629	636	661	674	2,600
SW&B as % of Revenue	54.2%	53.6%	55.7%	55.4%	54.8%
Adjusted Other Operating Expenses	177	172	172	176	697
Other OpEx as % of Revenue	15.3%	14.5%	14.5%	14.5%	14.7%
Supplies	54	52	55	56	216
Supplies as % of Revenue	4.6%	4.4%	4.6%	4.6%	4.6%
Occupancy Costs	14	14	13	13	54
Y/Y Growth			1.0%	1.0%	
General & Administrative (ex SBC)	34	38	33	33	138
Y/Y Growth			2.0%	2.0%	
Other Income	2	2	2	2	9
Other Income % of Revenue	0.2%	0.2%	0.2%	0.2%	0.2%
Equity in nonconsolidated affiliates	0	1	1	1	3
Equity in nonconsolidated affiliates % of Revenue	0.0%	0.1%	0.1%	0.1%	0.1%
Noncontrolling Interest	26	28	26	27	106
Noncontrolling Interest % of Revenue	2.2%	2.4%	2.2%	2.2%	2.2%
Adjusted EBITDA (ex-NCI)	**$229**	**$250**	**$230**	**$240**	**$948**
EBITDA (ex-NCI) Margins	19.7%	21.0%	19.4%	19.7%	20.0%

Salaries, Wages & Benefits: Modelled improvement in contract labour (decrease from 1.8% to 1.6%), expansion in EPOB from 3.38 to 3.4, 4% wage and benefit inflation, held sign on bonuses constant

EBITDA guidance range of $920 to $950, conservatively model $946 (consensus at $937) for FY23. Attractive setup greenlights beat, +4% raise from mid-point after Q2 beat by +15%

- EHC beat consensus EBITDA by +15% in Q2 '23 and raised FY23 guidance by +4% at mid-point. Management guides conservatively (Q1 '23 beat by +10% and narrowed guide range by +1% at mid-point), tougher comps in H2 '23
- I model FY23 EBITDA of $948 (+9% Y/Y) vs consensus forecast of $937, conservatively project EBITDA at high end of guidance driven by SW&B improvement and fixed cost leverage

Summary Projected P&L

Encompass Health

Fiscal year end December 31

Projected P&L for N3Y

	30-Sep-23 Q3E	31-Dec-23 Q4E	FY2023	31-Mar-24 Q1E	30-Jun-24 Q2E	30-Sep-24 Q3E	31-Dec-24 Q4E	FY2024	31-Mar-25 Q1E	30-Jun-25 Q2E	30-Sep-25 Q3E	31-Dec-25 Q4E	FY2025
Total Revenue	**$1,185**	**$1,216**	**$4,749**	**$1,240**	**$1,250**	**$1,252**	**$1,298**	**$5,039**	**$1,319**	**$1,330**	**$1,334**	**$1,384**	**$5,368**
Salaries, Wages and Benefits	661	674	2,600	682	680	705	723	2,789	708	725	753	773	2,958
SW&B as % of Revenue	55.7%	55.4%	54.8%	55.0%	54.4%	56.3%	55.7%	55.4%	53.7%	54.5%	56.4%	55.8%	55.1%
Adjusted Other Operating Expenses	172	176	697	180	181	182	188	731	191	193	193	201	778
Other OpEx as % of Revenue	14.5%	14.5%	14.7%	14.5%	14.5%	14.5%	14.5%	14.5%	14.5%	14.5%	14.5%	14.5%	15.3%
Supplies	55	56	216	57	57	58	60	232	61	61	61	64	247
Supplies as % of Revenue	4.6%	4.6%	4.6%	4.6%	4.6%	4.6%	4.6%	4.6%	4.6%	4.6%	4.6%	4.6%	4.6%
Occupancy Costs	13	13	54	14	14	13	13	54	14	15	13	13	55
Y/Y Growth	1.0%	1.0%		1.0%	1.0%	1.0%	1.0%		1.0%	1.0%	1.0%	1.0%	
General & Administrative (ex SBC)	33	33	138	35	39	33	34	141	36	40	34	35	144
Y/Y Growth	2.0%	2.0%		2.0%	2.0%	2.0%	2.0%	2.8%	2.0%	2.0%	2.0%	2.0%	2.7%
Other Income	2	2	9	2	2	2	3	10	3	3	3	3	10
Other Income % of Revenue	0.2%	0.2%	0.2%	0.2%	0.2%	0.2%	0.2%	0.2%	0.2%	0.2%	0.2%	0.2%	0.2%
Equity in nonconsolidated affiliates	1	1	3	1	1	1	1	4	1	1	1	1	4
Equity in nonconsolidated % of Revenue	0.1%	0.1%	0.1%	0.1%	0.1%	0.1%	0.1%	0.1%	0.1%	0.1%	0.1%	0.1%	0.1%
Noncontrolling Interest	26	27	106	27	27	28	29	111	29	29	29	30	118
Noncontrolling Interest % of Revenue	2.2%	2.2%	2.2%	2.2%	2.2%	2.2%	2.2%	2.2%	2.2%	2.2%	2.2%	2.2%	2.2%
Adjusted EBITDA (ex-NCI)	**$230**	**$240**	**$948**	**$248**	**$254**	**$238**	**$254**	**$994**	**$284**	**$271**	**$254**	**$272**	**$1,082**
EBITDA (ex-NCI) Margins	19.4%	19.7%	20.0%	20.0%	20.3%	19.0%	19.6%	19.7%	21.5%	20.4%	19.1%	19.7%	20.1%

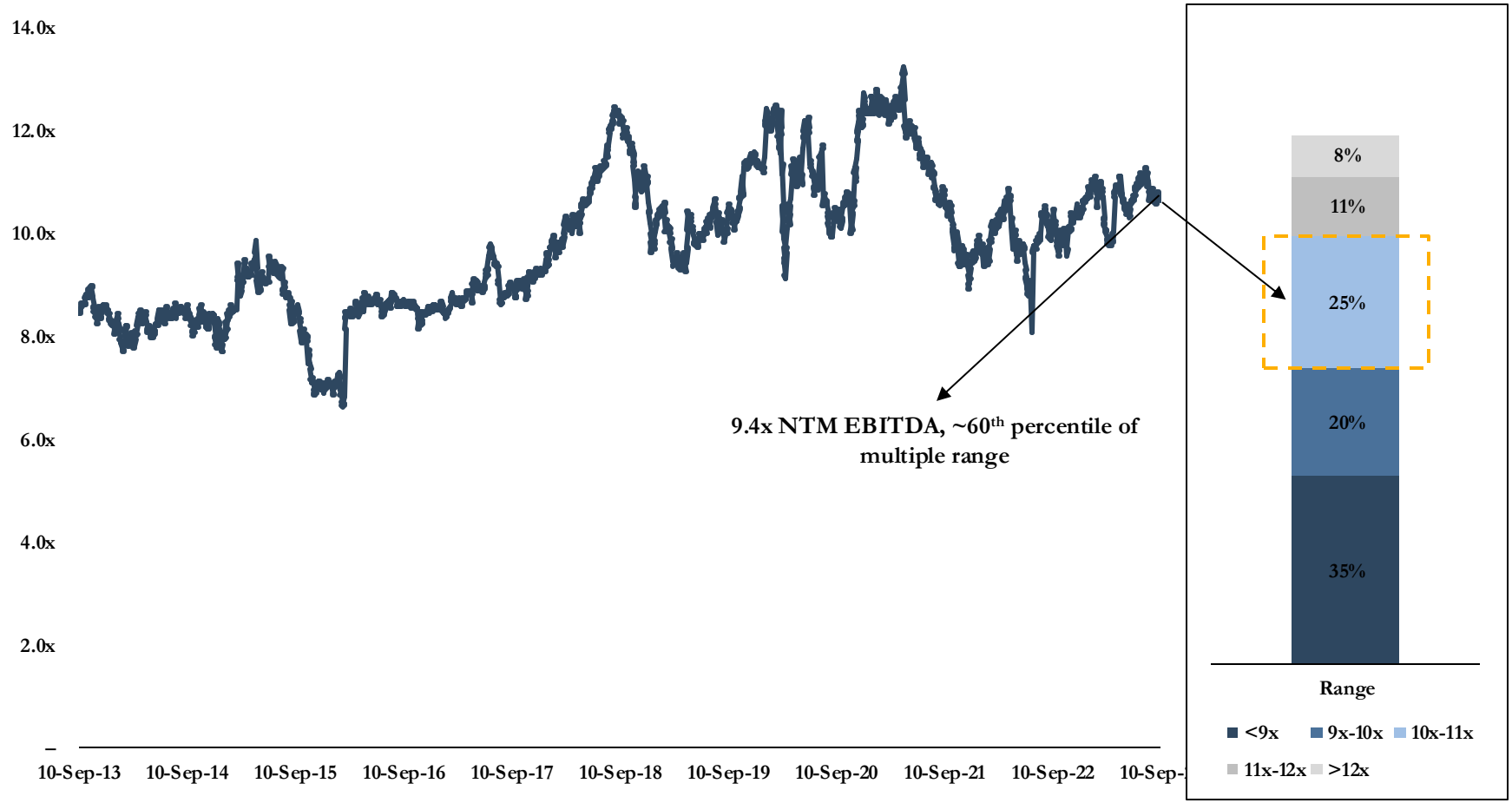
- Model 3-year EBITDA CAGR of 9%, discharge growth of ~5%
- As SW&B improvements are realized (contract labour easing, agency rates inflecting downwards), glide path to ~20-21% EBITDA – NCI margins is reasonable. EPOB normalizes to 3.40 for projection period
- EHC expects to realise leverage on fixed costs (SW&B inflation temporarily negates realized leverage), further adds to margin expansion story

Assessing the Multiple

Encompass Health

L10Y analysis indicates that EHC trades at 60th percentile of its multiple range

TEV/Forward EBITDA Multiple



Analysis at Various Prices

Encompass Health

EHC trades at 9.7x 2023E EBITDA and 9.2x 2024E EBITDA

Disaggregating EHC's Multiple

$ Millions		Current	Price Sensitivity									
Share Price		$63	$55	$60	$65	$70	$75	$80	$85	$90	$95	$100
% to Current		–	(13%)	(5%)	3%	10%	18%	26%	34%	42%	50%	58%
Shares Outstanding		100	100	100	100	100	100	100	100	100	100	100
Market Capitalization		**$6,355**	**$5,513**	**$6,014**	**$6,515**	**$7,016**	**$7,517**	**$8,018**	**$8,520**	**$9,021**	**$9,522**	**$10,023**
Plus: Debt		2,939	2,939	2,939	2,939	2,939	2,939	2,939	2,939	2,939	2,939	2,939
Plus: Minority Interest		0	0	0	0	0	0	0	0	0	0	0
Less: Cash		118	118	118	118	118	118	118	118	118	118	118
Total Enterprise Value		**$9,176**	**$8,334**	**$8,835**	**$9,336**	**$9,837**	**$10,339**	**$10,840**	**$11,341**	**$11,842**	**$12,343**	**$12,844**
Revenue												
2023E	$4,749	1.9x	1.8x	1.9x	2.0x	2.1x	2.2x	2.3x	2.4x	2.5x	2.6x	2.7x
2024E	5,039	1.8x	1.7x	1.8x	1.9x	2.0x	2.1x	2.2x	2.3x	2.3x	2.4x	2.5x
2023E - Street	4,781	1.9x	1.7x	1.8x	2.0x	2.1x	2.2x	2.3x	2.4x	2.5x	2.6x	2.7x
2024E - Street	5,179	1.8x	1.6x	1.7x	1.8x	1.9x	2.0x	2.1x	2.2x	2.3x	2.4x	2.5x
EBITDA - NCI												
2023E	$948	9.7x	8.8x	9.3x	9.8x	10.4x	10.9x	11.4x	12.0x	12.5x	13.0x	13.5x
2024E	994	9.2x	8.4x	8.9x	9.4x	9.9x	10.4x	10.9x	11.4x	11.9x	12.4x	12.9x
2023E - Street	937	9.8x	8.9x	9.4x	10.0x	10.5x	11.0x	11.6x	12.1x	12.6x	13.2x	13.7x
2024E - Street	1,012	9.1x	8.2x	8.7x	9.2x	9.7x	10.2x	10.7x	11.2x	11.7x	12.2x	12.7x

- EHC trades ~9.7x 2023E EBITDA, at 60th percentile of its multiple range
- Given the current setup (strong utilization, demographic trends, margin inflection, share capture), I think the current multiple is reasonable and see potential for expansion

What You Need to Believe

Encompass Health

Glide to path to 15% IRR if you believe current multiple holds and ~9% EBITDA CAGR

~9% EBITDA CAGR is Achievable

Valuation Date	10/11/2023
Share Price	$63.5
Shares Outstanding	100.2
Market Cap	$6,361.6
(+) Debt	2,938.8
(-) Cash	117.5
Enterprise Value	**$9,182.9**
NTM EBITDA vRM	$971.8
Implied EV/NTM EBITDA	9.4x
Target IRR	15.0%
Target Exit Date	10/31/2025
Exit Enterprise Value Proceeds	$12,144.4
Exit Multiple	10.5x
Implied NTM EBITDA on Exit	$1,156.6
Implied EBITDA CAGR	**8.8%**

~13% EBITDA CAGR is Within Reason

Valuation Date	10/11/2023
Share Price	$63.5
Shares Outstanding	100.2
Market Cap	$6,361.6
(+) Debt	2,938.8
(-) Cash	117.5
Enterprise Value	**$9,182.9**
NTM EBITDA vRM	$971.8
Implied EV/NTM EBITDA	9.4x
Target IRR	20.0%
Target Exit Date	10/31/2025
Exit Enterprise Value Proceeds	$13,223.4
Exit Multiple	10.5x
Implied NTM EBITDA on Exit	$1,259.4
Implied EBITDA CAGR	**13.4%**

Investment Risks

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Stroke of pen risk is a key alignment criteria for provider investments

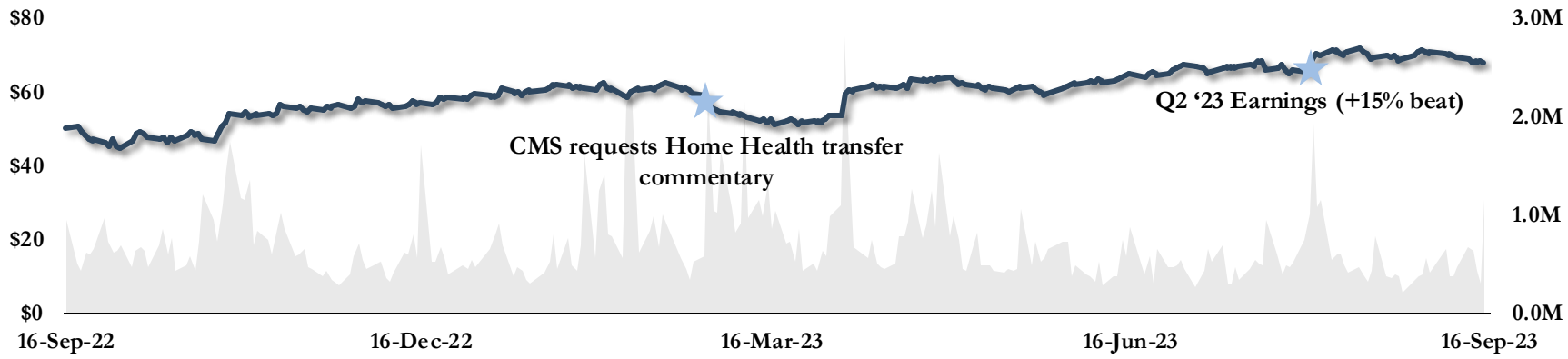
Stroke of Pen Risk	EHC is completely at the mercy of the CMS and MCOs when it comes to rate determination. As a result, a key drier of tEHCe stock is IRF PPS reimbursement updates. Investors gain comfort with stroke of pen risk since ~22% of industry is upside on margins, IRFs offer strong value proposition and payors want to keep them in their network, SW&B inflation damaged margins which can be restored by inflation linked price bumps. Sequestration impact on revenue per discharge cannot be mitigated
Salaries and Wages Grow in Excess of IRF PPS	Sequestration resumption in April 2022 increases pressure on IRF reimbursement. CMS rate update of +4% for FY23 and +3% for FY24 nets out to +2% and +1% respectively. Management guides to +3-4% SW&B for the balance of FY23, commentary indicates easing in inflation down to market basket rate update level. In case of SW&B running hotter than expected, EHC could experience continued margin pressure
Home Health Transfer Exemption Rule Change	Washington Analysis and Office of Inspector General report on IRF transfers to home-health agencies highlights ~$1B in savings for CMS in FY16 and FY17. CMS requested input from IRF providers alongside its FY23 PPS rate update. In the case of a rule change, estimates show that it would result in a net 6% price reduction. I think risk of complete overhaul is overblown since home health agencies aren't substitutes for IRF care. IRFs deal with specialized conditions and offer a strong value proposition, low likelihood of regulatory interference

Catalysts

Encompass Health

Look for moves on Investor Day commentary, favorable industry read through

Previous "Alpha" Days and Periods



My Event Path

1. **MCO Commentary:** UNH Earnings Call on 13th October 2023 indicated positive utilization trends in Medicare Advantage, stock reacted positively +1.41% at close

2. **Earnings Calls:** EHC stock reacted positively to Q2 '23 beat (+15%), expect further moves as FY23 guidance is conservative. Further, MCO commentary provides significant read-through on EHC volumes

3. **Share Gains and Utilization Momentum:** Long-term tailwinds of demographic growth and site of care shift underpin strong discharge growth. Sentiment shift as investors realize that EHC is the best way to play aging population trend

Ownership Breakdown

Encompass Health

Constructive ownership base with balance of "slow" and "fast money"

Holder	Common Stock Equivalent Held	% of Common Shares Outstanding	Market Value ($M USD)	Change in Common Stock	% Change in Common Stock	Position Date
The Vanguard Group, Inc.	9,515,573	9.494	595.6	(377,399)	(3.8%)	Jun-30-2023
BlackRock, Inc. (NYSE:BLK)	9,302,100	9.281	582.2	(419,819)	(4.3%)	Jun-30-2023
Wellington Management Group LLP	8,992,408	8.972	562.8	(189,647)	(2.1%)	Jun-30-2023
William Blair Investment Management, LLC	5,613,363	5.6	351.3	16,150	0.3%	Jun-30-2023
Invesco Ltd. (NYSE:IVZ)	2,974,720	2.968	186.2	1,104,932	59.1%	Jun-30-2023
State Street Global Advisors, Inc.	2,831,162	2.825	177.2	(40,543)	(1.4%)	Jun-30-2023
JP Morgan Asset Management	2,806,077	2.8	175.6	(79,368)	(2.8%)	Jun-30-2023
Massachusetts Financial Services Company	2,163,996	2.159	135.4	367,846	20.5%	Jun-30-2023
Point72 Asset Management, L.P.	1,793,504	1.789	112.3	560,982	45.5%	Jun-30-2023
Brown Advisory Incorporated	1,700,468	1.697	106.4	30,347	1.8%	Jun-30-2023
BNY Mellon Asset Management	1,683,104	1.679	105.3	(221,123)	(11.6%)	Jun-30-2023
Geode Capital Management, LLC	1,628,505	1.625	101.9	77,735	5.0%	Jun-30-2023
Janus Henderson Group plc (NYSE:JHG)	1,585,374	1.582	99.2	47,417	3.1%	Jun-30-2023
Dimensional Fund Advisors LP	1,531,489	1.528	95.9	(2,297)	(0.2%)	Jun-30-2023
Copeland Capital Management, LLC	1,271,681	1.269	79.6	44,696	3.6%	Jun-30-2023
FMR LLC	1,231,724	1.229	77.1	710,076	136.1%	Jun-30-2023
Nikko Asset Management Co., Ltd.	1,224,416	1.222	76.6	48,994	4.2%	Jun-30-2023
Magnetar Capital Partners, LP	1,129,648	1.127	70.7	(787,382)	(41.1%)	Jun-30-2023
Victory Capital Management Inc.	1,083,510	1.081	67.8	8,606	0.8%	Jun-30-2023
Schroder Investment Management North America Inc.	1,039,679	1.037	65.1	(24,458)	(2.3%)	Jun-30-2023
Citadel Advisors LLC	1,007,981	1.006	63.1	(114,111)	(10.2%)	Jun-30-2023
Norges Bank Investment Management	986,153	0.984	61.7	-	0.0%	Dec-31-2022
TimesSquare Capital Management, LLC	983,820	0.982	61.6	(161,390)	(14.1%)	Jun-30-2023
Kempen Capital Management NV	978,312	0.976	61.2	(89,430)	(8.4%)	Jun-30-2023
Paradice Investment Management Pty Ltd.	833,854	0.832	52.2	(16,729)	(2.0%)	Jun-30-2023
Northern Trust Global Investments	815,022	0.813	51.0	(7,906)	(1.0%)	Jun-30-2023
Millennium Management LLC	798,642	0.797	50.0	(206,125)	(20.5%)	Jun-30-2023
Capital Research and Management Company	787,849	0.786	49.3	-	0.0%	Jun-30-2023
Columbia Management Investment Advisers, LLC	777,263	0.775	48.6	(286,426)	(26.9%)	Jun-30-2023
Reinhart Partners, LLC	742,980	0.741	46.5	(3,608)	(0.5%)	Jun-30-2023
Toronto-Dominion Bank, Banking Investments	707,981	0.706	44.3	57,494	8.8%	Jun-30-2023
Junto Capital Management LP	698,497	0.697	43.7	698,497	New	Jun-30-2023
Charles Schwab Investment Management, Inc.	619,364	0.618	38.8	(2,809)	(0.5%)	Jun-30-2023
TD Asset Management, Inc.	617,758	0.616	38.7	19,399	3.2%	Jun-30-2023
Holocene Advisors, LP	574,171	0.573	35.9	556,060	3070.3%	Jun-30-2023
Leeward Investments, LLC	562,001	0.561	35.2	(27,031)	(4.6%)	Sep-30-2023